

STATE OF MICHIGAN THIRD JUDICIAL CIRCUIT WAYNE COUNTY	PERSONAL PROTECTION ORDER ☒ EX PARTE (DOMESTIC RELATIONSHIP)	CASE NO. 26-102221-PP
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Court address 1801 CAYMC, 2 Woodward Ave., Detroit, MI 48226

Court telephone no. 313-224-0120

ORI
MI-

Petitioner's name Adrienne Rockenhaus	v	Respondent's name, address, telephone no. Conrad Rockenhaus No Known Address					
Address and telephone no. where court can reach petitioner 35560 Grand River AVE UNIT #244 FARMINGTON HILLS, MI 48335 Cell: 734-585-6277							
Full name of respondent (type or print)* Conrad Rockenhaus		Driver's license number (if known)					
Height 6 Ft. 0 In.	Weight 190 Lbs.	Race* White	Sex* Male	DOB or age* 02/03/1982	Hair color Brown	Eye color Brown	Other identifying information fence on fire tattoo on forearm, upper arm tattoos

*These items **must** be filled in for the police/sheriff to enter on LEIN; the other items are not required but are helpful. **Needed for NCIC entry

Date: 02/20/2026 Judge: PPO Docket Judge P44565

1. This order is entered ☒ without a hearing. ☐ **after hearing.

THE COURT FINDS:

- ☒ 2. A petition requested respondent be prohibited from entry onto the premises, and either the parties are married, petitioner has a property interest in the premises, or respondent does not have a property interest in the premises.
- ☒ 3. Petitioner requested an ex parte order, which should be entered without notice because irreparable injury, loss, or damage will result from the delay required to give notice or notice itself will precipitate adverse action before the order can be issued
- ** ☒ 4. Respondent poses a credible threat to the physical safety of petitioner and/or a child of petitioner.
- ☐ 5. The respondent ☒ **is the spouse or former spouse of petitioner, had a child in common with petitioner, or is residing or had resided in the same household as petitioner. ☐ has or had a dating relationship with the petitioner.

IT IS ORDERED

6. Conrad Rockenhaus is prohibited from:
- Name
- ☒ a. entering onto property where petitioner lives.
- ☐ b. entering onto property at .
- ** ☒ c. assaulting, attacking, beating, molesting, or wounding Adrienne Rockenhaus .
- Name
- ☐ d. removing minor children from petitioner who has **legal** custody, except as allowed by custody or parenting-time order provided removal of the children does not violate other conditions of this order. An existing custody order is dated . An existing parenting-time order is dated .

Approved, SCAO

Distribute form to:

Form CC 376, Rev. 3/23

Court

MCL 600.2950, MCR 3.706, 18 USC 922(g)(8) (c)

Law enforcement agency

Page 1 of 2

Petitioner

Return

IT IS ORDERED:

6. (continued)

- ** ☒ e.** stalking as defined by MCL 750.411h and MCL 750.411i, which include but it not limited to:
- ☒ following or appearing within sight of the petitioner.
 - ☒ appearing at workplace/residence of the petitioner.
 - ☒ approaching or confronting petitioner in a public place or on private property.
 - ☒ entering onto or remaining on property owned, leased, or occupied by petitioner.
 - ☒ sending mail/other communications to the petitioner.
 - ☒ contacting the petitioner telephone.
 - ☒ placing an object on or delivering an object to property owned, leased, or occupied by petitioner.
- ☐ f. interfering with petitioner's efforts to remove his/her children/personal property from premises solely owned or leased by respondent.
- ** ☒ g.** threatening to kill or physically injure Adrienne Rockenhaus.
- ☒ h. interfering with petitioner at his/her place of employment or education or engaging in conduct that impairs his/her employment or educational relationship or environment
- ☐ i. having access to information in records concerning a minor child of petitioner and respondent that will reveal petitioner's address, telephone number, or employment address or that will reveal the child's address or telephone number.
- ☒ j. intentionally causing petitioner mental distress or exerting control over petitioner by:
- ☒ injuring, killing, torturing, or neglecting, or threatening to injure, kill torture, or neglect any animal in which petitioner has an ownership interest
 - ☒ removing any animal from his/her possession in which petitioner has an ownership interest.
 - ☒ retaining or obtaining possession of any animal in which petitioner has an ownership interest.
- ** ☒ k.** purchasing or possessing a firearm.
- ☐ l. other: _____
7. As a result of this order, federal and/or state law may prohibit you from possessing or purchasing ammunition or a firearm.
8. Violation of this order subjects respondent to immediate arrest and to the civil and criminal contempt powers of the court. If found guilty, respondent shall be imprisoned for not more than 93 days and may be fined not more than \$500.00.
9. **This order is effective when signed, enforceable immediately, and remains in effect until 02/20/2027**.
This order is enforceable anywhere in this state by any law enforcement agency when signed by a judge, and upon service, may also be enforced by another state, and Indian tribe, or a territory of the United States. If respondent violates this order in a jurisdiction other than this state, respondent is subject to enforcement and penalties of the state, Indian tribe, or United States territory under whose jurisdiction the violation occurred.
10. The court clerk shall file this order with MSP who will enter it into the LEIN.
Name of law enforcement agency
11. Respondent may file a motion to modify or terminate this order. If this is an ex parte order, a motion must be filed within 14 days after being served with or receiving actual notice of the order. Forms and instructions are available from the clerk of court.
12. A motion to extend the order must be filed 3 days before the expiration date in item 9 or a new petition must be filed.

J. A. Miller Martin
Judge signature and date

02/20/2026

PROOF OF SERVICE

TO PROCESS SERVER: You must serve the personal protection order and file proof of service with the court clerk. If you are unable to complete service, you must return this original and all copies to the court clerk..

CERTIFICATE OF SERVICE/NONSERVICE

- ☐ I served ☐ personally ☐ by registered or certified mail, return receipt requested, and delivery restricted to the respondent (copy of return receipt attached) a copy of the personal protection order, together with the attachments listed below, on:
- ☐ I have attempted to serve a copy of the personal protection order, together with the attachments listed below, and have been unable to complete service on:

Respondent's name	Date and time of service
Place or address of service	
Attachments (if any)	

- ☐ I am a sheriff, deputy sheriff, bailiff, appointed court officer or attorney for a party.
- ☐ I am a legally competent adult who is not a party or an officer of a corporate party. I declare under the penalties of perjury that this certificate of service has been examined by me and that its contents are true to the best of my information, knowledge, and belief.

Service fee \$	Miles traveled	Fee	
Incorrect address fee	Miles traveled	Fee \$	TOTAL FEE \$

Signature_____
Name (type or print)**ACKNOWLEDGEMENT OF SERVICE**

I acknowledge that I have received service of a copy of the personal protection order, together with.

Attachments (if any)_____
Date and time_____
Respondent's signature_____
Name (type and print)

These forms were emailed from MLH-Forms via a tool developed and maintained by Michigan Legal Help.

Petitioner: Adrienne Rockenhaus; Email:
adrienne@cannabytes.net

STATE OF MICHIGAN 3rd JUDICIAL CIRCUIT WAYNE COUNTY	PETITION FOR PERSONAL PROTECTION ORDER (DOMESTIC RELATIONSHIP)	CASE NO. and JUDGE
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Court address

2 Woodward Avenue, Detroit, MI 48226

Court telephone no.

(313) 224-5261

A Petitioner's name Adrienne Rockenhaus Address and telephone no. where court can reach petitioner 35560 Grand river ave, Unit 244 farmington hills, MI 48335 (734) 585-6277	Age 46	Respondent's name, address, and telephone no. Conrad Rockenhaus PO Box 1000, Inmate Number 39400-480 Milan, MI 48160 Unknown phone	Age 44
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- B** 1. The petitioner and respondent ☒ are married to each other. ☐ were married to each other.
☐ have a child in common. ☐ have or had a dating relationship. ☒ reside or resided in the same household.

- C** 2. ☐ The respondent is required to carry a firearm in the course of his/her employment. ☐ Unknown.

- D** 3. a. There ☐ are ☒ are not other pending actions in this or any other court regarding the parties.

Case number	Name of court, county, and state or province	Name of judge
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- b. There ☐ are ☒ are not orders/judgments entered by this or any other court regarding the parties.

Case number	Name of court, county, and state or province	Name of judge
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- E** 4. I need a personal protection order because: Explain what has happened. Attach separate sheet(s).

Please see attached "Statement of Facts".

- F** 5. I ask the court to grant a personal protection order prohibiting the respondent from:

- ☒ a. entering onto the property where I live. I state that either I have a property interest in the premises, I am married to the respondent, or the respondent has no property interest in the premises.

- ☐ b. entering onto the property at _____
Address

- ☒ c. assaulting, attacking, beating, molesting, or wounding Adrienne Rockenhaus
Name(s)

- ☐ d. removing the minor children from the petitioner who has **legal** custody, except as allowed by a custody or parenting time order as long as removal of the children does not violate other conditions of the personal protection order.

- ☒ e. stalking as defined under MCL 750.411h and MCL 750.411i, which includes but is not limited to:
☒ following me or appearing within my sight. ☒ appearing at my workplace or residence.
☒ sending mail or other communications to me. ☒ contacting me by telephone.
☒ approaching or confronting me in a public place or on private property.
☒ entering onto or remaining on property owned, leased, or occupied by me.
☒ placing an object on or delivering an object to property owned, leased, or occupied by me.

- ☐ f. interfering with efforts to remove my children/personal property from premises solely owned/leased by the respondent.

- ☒ g. threatening to kill or physically injure Adrienne Rockenhaus .
- ☒ h. interfering with me at my place of employment or education or engaging in conduct that impairs my employment or educational relationship or environment.
- ☐ i. having access to information in records concerning a minor child of mine and the respondent that will reveal my address, telephone number, or employment address or that will reveal the child's address or telephone number.
- ☒ j. intentionally causing me mental distress or exerting control over me by:
- ☒ injuring, killing, torturing, or neglecting, or threatening to injure, kill, torture, or neglect any animal in which I have an ownership interest.
 - ☒ removing any animal from my possession in which I have an ownership interest.
 - ☒ retaining or obtaining possession of any animal in which I have an ownership interest.
- ☒ k. purchasing or possessing a firearm.
- ☒ l. other: Please see "Relief Requested" in Attachment. .

G 6. I make this petition under the authority of MCL 600.2950/MCL 600.2950a and ask the court to grant a personal protection order.

- ☒ I request an ex parte order because immediate and irreparable injury, loss, or damage will occur between now and a hearing or because notice itself will cause irreparable injury, loss, or damage before the order can be entered.

H ☐ 7. I have a next friend petitioning for me. I certify that the next friend is not disqualified by statute and is an adult.

I February 19, 2026
Date

/s/ Adrienne Rockenhaus

Petitioner's/Next friend's signature

Adrienne Rockenhaus

Name (type or print)

(e-signed as allowed by MCR 1.109)

**Petition for Personal Protection Order
(Domestic)**

Case No.

STATEMENT OF FACTS

4. (Continued)

Petitioner alias: Adrienne Blair, Adrienne Hein.

Respondent alias: Rock Rockenhaus.

The parties do not currently reside together.

The parties' relationship is as follows: Spouses.

Other children living in Petitioner's home who are not the children of Respondent: None.

Other children living in Respondent's home who are not the children of Petitioner: None.

----- Incidents -----

a. One incident of violence, threat of violence, or other abusive behavior by Conrad Rockenhaus occurred on or about This incident occurred on or about February 18, 2026, when the Respondent filed public documents designed to legally silence me, isolate me, and strip me of protections immediately after I reported his history of sexual abuse and coercion to his legal counsel and medical providers at the following location: Electronically over the BOP CorrLinks Email System and the Federal Court Public Docket (PACER).

At that time, Conrad Rockenhaus said or did the following: On February 9, 2026, I officially informed the Respondent's attorney that I was revoking his residence at my home due to his history of physical and sexual abuse, and that I would not pick him up upon his March 2, 2026 release from prison. On February 12, I provided his legal and medical teams with digital receipts proving he had secretly stockpiled high-dose Viagra to chemically facilitate sexual abuse against me in early 2025 while I was medically incapacitated. Immediately after I reported this sexual abuse, the Respondent began a campaign of retaliatory coercive control. On February 14, 2026, he used the prison's electronic communication system to send a cyberstalking taunt to my private email, using a mocking alias to terrorize me regarding my past trauma. Four days later, on February 18, 2026, he escalated this harassment by filing retaliatory public court documents designed to legally silence me. In these filings, he falsely claimed he had revoked my Power of Attorney weeks prior and explicitly demanded that the court 'no longer accept any filings' from me. This was a calculated act of litigation harassment designed to strip away my legal protections and isolate me just days before his release.

His public filings directly contradict his private manipulation. While publicly claiming he 'fired' me to silence my reports of abuse, he privately instructed his attorney to demand that I pick him up and house him. Because he is medically unstable with a Traumatic Brain Injury, has no independent housing, and is actively using federal government systems to stalk and punish me for exposing his sexual predation, I am in extreme, reasonable fear that he will attempt to ambush my home and force entry immediately upon his release on March 2nd.

Because of this incident I felt completely terrorized, intimidated, and in severe fear for my physical safety. I

felt legally trapped and psychologically tortured by his obsessive need to control and punish me from his prison cell. His escalating harassment triggered severe panic, hypervigilance, and overwhelming anxiety, which my psychiatrist (Dr. Robert Garcia) has formally diagnosed as Severe PTSD resulting directly from these types of adversarial attacks. I felt profound dread and helplessness knowing that he is intentionally trying to strip away my legal protections just days before his March 2nd release, leaving me terrified that he will ambush my home to retaliate against me for exposing his history of sexual and physical abuse.

b. Another incident of violence, threat of violence, or other abusive behavior by Conrad Rockenhaus occurred on or about This incident occurred on or about October 13, 2025, when the Respondent orchestrated a campaign of proxy stalking and harassment by directing a third-party associate to bombard my phone after I had cut off communication for my safety at the following location: My private cell phone / Electronic Communication.

At that time, Conrad Rockenhaus said or did the following: On October 13, 2025, the Respondent utilized the network of a 7-count indicted child predator (Jorj Michael Austin) to bypass my communication block. He directed an associate, Jerome Sallad, to bombard me with nine back-to-back phone calls and threatening text messages (stating 'This for your husband he's not mad') to force contact. This proxy harassment was a calculated attempt to terrorize me and prove that he could still reach me through dangerous individuals even after I told him to stop.

Because of this incident I felt Terrified, harassed, and violated. It proved he would expose me to dangerous criminals just to exert control over me.

c. Another incident of violence, threat of violence, or other abusive behavior by Conrad Rockenhaus occurred on or about In early 2025, while I was severely ill and recovering from a violent physical assault by a former landlord, I explicitly revoked sexual consent to protect my body while I healed. The Respondent did not respect this boundary. Instead, he aggressively coerced me for sex until the physical and emotional stress caused me to sob and physically collapse. Instead of rendering medical aid to his collapsed wife, I woke up to him raping me while I was unconscious. I have since recovered digital pharmacy receipts proving that after I explicitly withdrew consent, he was 'double-filling' and secretly stockpiling maximum-strength 100mg Sildenafil (Viagra) through a private telehealth app on February 3, 2025, while simultaneously obtaining it from the VA. He used these stockpiled drugs to chemically engineer his arousal and override my explicit 'no' while I was traumatized and physically unable to fight back. This was calculated, predatory sexual violence at the following location: 2468 Fernwood Ave Ann Arbor, MI 48104.

At that time, Conrad Rockenhaus said or did the following: In early 2025, I was severely ill and recovering from a violent physical assault by a former landlord and toxic mold poisoning from my old apartment. Because of this medical incapacitation and trauma, I explicitly revoked sexual consent to protect my body while I healed. The Respondent did not respect this boundary. Instead, he aggressively coerced and bullied me for sex until the physical and emotional stress caused me to sob and physically collapse from exhaustion. Instead of rendering medical aid to his collapsed wife, I woke up to find him raping me while I was unconscious. I have since recovered digital pharmacy receipts proving that after I explicitly withdrew consent, he was 'double-filling' and secretly stockpiling maximum-strength 100mg Sildenafil (Viagra) through a private telehealth app on February 3, 2025, while simultaneously obtaining it from the VA. He deliberately used these stockpiled drugs to chemically engineer his arousal and override my explicit 'no' while I was traumatized and physically unable to fight back. This was a calculated act of predatory sexual violence.

Because of this incident I felt completely violated, physically terrified, and profoundly betrayed. It was horrifying to realize that he viewed my severe medical incapacitation and physical collapse not as a reason to render aid, but as an opportunity to prey on me while I was unconscious. The discovery that he secretly stockpiled chemical drugs to manufacture his ability to override my explicit revocation of consent left me feeling hunted and helpless in my own sanctuary. It proved to me that he is a dangerous sexual predator who lacks all basic human empathy, leaving me in absolute, paralyzing terror of what he will physically do to me if he is ever allowed near me again.

As a result, I was physically injured: Severe physical trauma and bodily violation resulting from criminal sexual conduct (rape) committed against me while I was medically incapacitated and unconscious.

d. Another incident of violence, threat of violence, or other abusive behavior by Conrad Rockenhaus occurred on or about This incident occurred on or about May 4, 2023, when the Respondent became physically aggressive toward me and forcibly grabbed my phone to prevent me from calling 911 for emergency help at the following location: 42580 POSTIFF AVE APT 39 PLYMOUTH, MI 48170.

At that time, Conrad Rockenhaus said or did the following: On May 4, 2023, the Respondent became highly unstable and physically aggressive toward me in our home. When I attempted to call 911 for help, he physically pushed me and forcibly grabbed the phone out of my hand to silence me and isolate me. Because he had physically overpowered me and taken my device, I was forced to use a voice-activated command ('Hey Google') to successfully summon emergency services while he actively tried to prevent me from getting help. When the Plymouth Police arrived and assessed the danger, they recognized the threat and issued him a strict ultimatum: 'Go to Jail or Go to the Hospital.' He chose to be transported to the VA hospital to avoid being arrested for his physical aggression and interference with an emergency communication.

Because of this incident I felt completely trapped, helpless, and terrified for my life. When he physically overpowered me and forcibly took my phone out of my hand, I realized he was willing to use physical violence to isolate me and stop me from getting emergency police protection. Because he intentionally severed my lifeline to the outside world, I felt profoundly intimidated and hunted in my own home. This incident proved that he is a severe physical danger to me when he becomes unstable, leaving me in absolute, permanent fear of what he will do if he is ever permitted to return to my residence.

As a result, I was physically injured: I sustained a visible scratch on my face during the physical struggle when the Respondent physically overpowered me and forcibly grabbed my phone. I took a photograph of the physical injury for my records, but the Respondent later accessed my personal phone without my consent and deliberately deleted the photograph to destroy the evidence of his assault and cover up his actions.

The police were called and did the following: The responding officers recognized the physical danger, acknowledged a crime had occurred regarding the 911 interference, and told me they had grounds for an arrest. A female officer specifically gave me the choice of whether to send him to jail or have him transported to the hospital for his severe instability. Believing he was in a genuine medical crisis, I chose the hospital in good faith to get him psychiatric help. The police then issued him a strict ultimatum of 'Go to jail or go to the hospital.' They supervised his removal from the home and had him transported by ambulance to the Ann Arbor VA Hospital.

e. Another incident of violence, threat of violence, or other abusive behavior by Conrad Rockenhaus occurred on or about This incident occurred on or about various dates between 2023 and 2025, when the Respondent subjected me to multiple other instances of unrecorded physical assault and aggression behind closed doors at the following location: Plymouth apartment, Ann Arbor home, Electronic communication.

At that time, Conrad Rockenhaus said or did the following: Between 2023 and 2026, the Respondent subjected me to a continuous pattern of physical aggression, coercion, and domestic violence behind closed doors. Because of his escalating volatility, he would frequently use his physical size to overpower, intimidate, and trap me when I tried to set basic safety boundaries. I did not call the police for every instance of being pushed or physically overpowered because the system had previously minimized his violence, which taught him he could hurt me without consequence. His physical intimidation created an environment of absolute coercive control, leaving me completely paralyzed with fear. When he was incarcerated in late 2025, this pattern of coercive control did not stop; it simply shifted mediums. He weaponized the prison's CorrLinks email system to subject me to continuous psychological abuse, including sending barrages of abusive, demanding messages to punish me when I was medically vulnerable with a head injury and physically unable to answer his demands. The specific incidents I have provided in this petition are simply the documented peaks of a much larger, multi-year cycle of physical and psychological terror that will immediately resume if he is allowed near me upon his release.

Because of this incident I felt completely broken, trapped, and permanently terrorized. Living under his continuous physical intimidation and coercive control left me completely paralyzed with fear. When the system minimized his early violence, it taught me that asking for help was useless, forcing me to suffer his escalating abuse in silence as a survival tactic. When his physical abuse shifted to relentless psychological torture and

digital harassment from prison while I was recovering from a head injury, I realized I could never truly escape him. This years-long, inescapable campaign of terror has resulted in a formal medical diagnosis of severe PTSD, leaving me in a constant state of hypervigilance and in absolute, paralyzing dread of what he will do to me if this order is not signed today.

As a result, I was physically injured: During these various incidents, I sustained physical injuries, including soreness, bruising, and physical pain from being forcibly grabbed, pushed, and physically overpowered by the Respondent. I do not have photographic evidence or medical reports for these specific injuries because the VA and law enforcement had previously minimized his physical violence against me in May 2023. That systemic failure taught me that documenting or reporting my injuries would only result in me being gaslit and blamed. Enduring these physical injuries and bruises in silence became a necessary survival tactic to avoid triggering his escalating volatility.

f. Another incident of violence, threat of violence, or other abusive behavior by Conrad Rockenhaus occurred on or about This incident occurred on or about two separate occasions in 2024, when the Respondent maliciously weaponized law enforcement against me by making false reports to the police to trap me and discredit me at the following location: 2468 Fernwood Ave Ann Arbor, MI 48104.

At that time, Conrad Rockenhaus said or did the following: On two separate occasions in 2024, I fled our residence to escape the Respondent's escalating volatility and abuse. To punish me for escaping and to reassert his control, the Respondent maliciously called 911 and filed false reports claiming that I was suicidal and intended to commit 'suicide by cop.' Because of his calculated lies, responding officers intercepted me, detained me, patted me down, and treated me like a dangerous criminal while I was actively fleeing domestic abuse. He deliberately weaponized the police to traumatize me, destroy my credibility with law enforcement, and ensure I would be too terrified to ever call 911 for help when he assaulted me in the future.

Because of this incident I felt completely helpless, discredited, and terrified of the very people who were supposed to protect me. Realizing he could successfully manipulate the police into targeting me instead of him made me feel like there was absolutely no escape from his abuse. It left me with profound trauma and a permanent fear that if I ever called the authorities for help, he would simply lie and have me arrested or committed.

The police were called and did the following: The police were called by the Respondent. Operating on his false, malicious reports that I was suicidal and seeking 'suicide by cop,' the responding officers intercepted me while I was actively fleeing his abuse on foot. Because of his calculated lies, the police detained me, physically patted me down, and treated me like a dangerous criminal rather than a victim escaping a highly volatile environment. By weaponizing the police to interrogate and detain me, he successfully discredited me and achieved his goal of making me completely terrified to ever seek help from law enforcement again.

g. I would also like the court to know: The Respondent poses an imminent, life-threatening danger to me upon his scheduled release from federal prison on March 2, 2026. He is a 100% disabled veteran suffering from a severe, untreated Traumatic Brain Injury (TBI) and a seizure disorder. He is currently being administered a volatile combination of psychotropic drugs and Suboxone, which is the exact medication class that fueled his physical aggression toward me in May 2023. This danger is not speculative: following a previous release from federal custody in October 2022, he suffered an immediate catastrophic decompensation and attempted suicide within one week, requiring over 60 days of inpatient VA hospitalization. He has a history of extreme mental instability and suicidality, including a 2023 psychiatric hospitalization after he laid on railroad tracks awaiting an oncoming train. Beyond physical violence, he exercises profound coercive control through financial abuse, medical deception, and the 'facilitation of violence'. He has dissipated over \$150,000 of our family assets due to his TBI-driven impulsivity. In September 2024, after I was physically assaulted by a landlord, the Respondent knowingly lied to me about securing legal counsel, deceiving me into believing I was safe and effectively trapping me in a residence with a violent aggressor. He also pathologically lies about his medical stability; prior to his arrest, he spent 11 months deceiving me and his Probation Officer into believing he was attending VA psychiatric appointments when he had actually abandoned his care. From prison, he continues to file retaliatory legal motions specifically designed to discredit and silence me. When he cannot physically reach me, he utilizes proxies, previously outsourcing his harassment by giving my private contact information to the associates of indicted criminals to bombard me with threats and

bypass my safety boundaries.

He is currently operating under severe medical delusions, believing he has a safe release plan when he is actually indigent and effectively homeless. Because he is emerging from a multi-year cycle of untreated head trauma, chemical volatility, and escalating retaliation, he is entirely unpredictable. I am in absolute, paralyzing fear for my life and safety if this order is not in place before the prison gates open.

STATE OF MICHIGAN 3rd JUDICIAL CIRCUIT WAYNE COUNTY	ATTACHMENT TO PETITION FOR PERSONAL PROTECTION ORDER	CASE NO.
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Court address ORI MI-	2 Woodward Avenue, Detroit, MI 48226	Court telephone no. (313) 224-5261
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Petitioner's name Adrienne Rockenhaus					Respondent's name, address, telephone no., and driver's license no. Conrad Rockenhaus PO Box 1000, Inmate Number 39400-480 Milan, MI 48160 Unknown phone		
Address and telephone no. where court can reach petitioner 35560 Grand river ave, Unit 244 farmington hills, MI 48335 (734) 585-6277							
Height 6 ft, 0 in	Weight 190	Race* White	Sex* M	Date of birth or age* February 3, 1982	Hair color Brown	Eye color Brown	Other identifying information fence on fire tattoo on forearm, upper arm tattoos

*These items **must** be filled in for the police/sheriff to enter on LEIN; the other items are not required but are helpful. **Needed for NCIC entry

Petition for Personal Protection Order (Domestic Relationship) for Adrienne Rockenhaus Attachment

Relief Requested – Other Relief, continued from F5I:

FINANCIAL HARASSMENT AND COERCIVE CONTROL: The Respondent is strictly prohibited from altering, changing, or redirecting direct deposit designations for federal VA benefits, survival funds, or joint banking accounts. He has a history of severe financial abuse, having dissipated our assets, and currently owes me over \$100,000, which includes a formally documented loan he took from my trust. Because I am filing this petition as an immediate ex parte emergency to secure my physical safety prior to his March 2nd release, I have not attached the voluminous financial receipts today, but I can provide them to the Court upon request. His threats to cut off my access to survival funds are a mechanism of psychological warfare and coercive control designed to terrorize me and interfere with my personal liberty.

DIGITAL WORKPLACE AND INFRASTRUCTURE PROTECTION: As the owner of a digital software and web business (Cannabytes, LLC), my 'workplace' consists of my digital infrastructure, servers, and domains (including cannabytes.net and rockenhaus.com). The Respondent has recently weaponized my workplace infrastructure to stalk me. On February 12 and 14, 2026, he deliberately routed targeted harassment messages through my business domains to bypass my personal boundaries. I am in extreme fear that upon his release, he will attempt to digitally hijack, sabotage, or interfere with my business domains and servers, or contact my clients to exert financial abuse and coercive control. I am requesting the court explicitly prohibit the Respondent from accessing, interfering with, or utilizing my business domains and prohibit him from contacting any clients or impairing my digital work environment in any way.

STATE OF MICHIGAN 3RD JUDICIAL CIRCUIT WAYNE COUNTY	EXHIBITS FOR PERSONAL PROTECTION ORDER (Domestic)	CASE NO.
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Exhibit A: Dr. Letter..... 2

Exhibit B: Exhibit_B_Evidence_of_Digital_Stalking_and_Taunts.pdf.....3

Exhibit C: Exhibit_A_Evidence_of_Sexual_Coercion.pdf.....9

Exhibit D: Exhibit_C_Evidence_of_Coercive_Control.pdf.....23

Exhibit E: Exhibit_D_Medical_Evidence_of_Prior_Violence.pdf.....30

PSYCHIATRY



PSYCHOLOGY

THE PSYCHIATRY AND PSYCHOLOGY CENTER

650 GRISWOLD ST, NORTHVILLE, MI 48167
PHONE (248) 912-0080
FAX (248) 912-0208

December 1st, 2025

RE: Adrienne Rockenhaus
DOB: 08/16/1979

To the Honorable United States District Court,

I am Dr. Robert Garcia, MD, a board-certified psychiatrist practicing in Northville, Michigan. I am currently the treating psychiatrist for Adrienne Rockenhaus, who is under my ongoing care at The Psychiatry & Psychology Center.

Adrienne has a longstanding history of anxiety and trauma-related symptoms, which have been further intensified by recent traumatic experiences. She currently exhibits PTSD-like symptoms including:

- Severe stress responses during conflict or procedural demands
- Difficulty maintaining emotional regulation under pressure
- Episodes of overwhelming anxiety that interfere with her ability to communicate or advocate for herself effectively

These symptoms are chronic, and they are notably exacerbated in high-stakes or adversarial environments such as legal proceedings. Based on my clinical assessment, it is my professional opinion that Adrienne is not able to safely represent herself in a federal civil matter. The stress and demands of self-representation would likely result in significant deterioration of her mental health and could worsen her symptoms.

For these reasons, I recommend that Adrienne be appointed legal counsel to ensure her ability to participate in legal proceedings without undue risk to her mental health.

Sincerely,

A handwritten signature in black ink, appearing to read 'R Garcia', written in a cursive style.

Robert Garcia, MD
Diplomate of the American Board of Psychiatry & Neurology
Fellow of the American Psychiatric Association
An affiliate of Henry Ford/Ascension Health Medical Staff

To the Honorable Judge:

The attached screenshots document the Respondent's recent and ongoing campaign of cyberstalking and psychological terror, conducted from inside a federal prison facility. Because the Respondent cannot physically reach me, he is weaponizing the Bureau of Prisons' "CorrLinks" email system to bypass my "no-contact" boundaries and infiltrate my private business network.

1. The "Sev Bivens" Harassment (February 14, 2026) On Valentine's Day (Feb 14, 2026), the Respondent sent an email invite to my private business domain (sevbivens@cannabytes.net).

- The Threat: "Bivens" is a direct, mocking reference to a federal *Bivens* Civil Rights Lawsuit I recently filed against a federal officer who sexually harassed me and assaulted my home.
- The Retaliation: The Respondent was permitted to use the government email system to send this stalking taunt just 48 hours after I formally notified his federal defense attorney that I was a victim of his sexual coercion and that I was filing for this exact Personal Protection Order. Instead of his communication privileges being locked down to protect a victim, he was allowed to use the prison system to retaliate against my request for safety.
- The Intent: By creating the alias "Sev Bivens," the Respondent is intentionally mocking my sexual assault trauma and signaling that he finds my victimization amusing. This was a calculated act of sadism designed to cause severe emotional distress.

2. The "Bre Newman" Harassment (February 12, 2026) Two days prior, the Respondent sent a similar invite to brenewmang@cannabytes.net.

- The Threat: He purposefully routed a message meant for an ex-partner from twenty years ago through *my* private business server's "catch-all" email system.
- The Intent: This was a digital "signal flare" intended to punish me, violate my digital workplace infrastructure, and assert coercive control.

Conclusion for the Court: Under MCL 750.411h, this constitutes unconsented contact that serves no legitimate purpose other than to harass, terrorize, and intimidate me. He is actively impairing my digital work environment and proving that physical distance will not stop his abuse.



Adrienne Rockenhaus <adrienne@rockenhaus.com>

Conrad's release & request

Kaycee Berente <Kaycee_Berente@fd.org>
To: "adrienne@rockenhaus.com" <adrienne@rockenhaus.com>

Fri, Jan 30, 2026 at 12:08 PM

Hi Adrienne,

I'm reaching out again on behalf of Conrad. His release plan is being set into motion now. Currently, Conrad and his unit team are working with veterans justice outreach services to set up a release plan that involves planning appointments with social workers and health care. But without your communication, Conrad is unsure if you will be picking him up or if he needs to make alternative arrangements to solidify his release plan.

Best,

Kaycee Berente

Assistant Federal Public Defender

Appellate Unit

Southern District of Ohio

10 W Broad St, Suite 1020

Columbus, OH 43215

Office: 614-469-2999

Direct: 614-469-4130

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From: Kaycee Berente
Sent: Thursday, January 29, 2026 11:35 AM
To: adrienne@rockenhaus.com
Subject: Conrad's release & request

00004

Hi Adrienne,

As a favor, Conrad has asked me to reach out to you in the hopes of opening communication back up between the two of you. He would like to discuss any filings made on his behalf with the courts- as anything filed in his name would require his authorization since he has not been deemed incompetent and although he is incarcerated, he still has access to the courts. He also would like to discuss his release, which is currently set for March 2nd, as Conrad will need a release plan.

I hope you are hanging in there and staying warm,

Kaycee Berente

Assistant Federal Public Defender

Appellate Unit

Southern District of Ohio

10 W Broad St, Suite 1020

Columbus, OH 43215

Office: 614-469-2999

Direct: 614-469-4130

This e-mail contains privileged & confidential information intended only for the use of the addressee(s) named above. If you are not the intended recipient of this e-mail, or an unauthorized employee or agent for delivering it to the intended, you are hereby notified that any dissemination or copying of this e-mail is strictly prohibited. If you have received this e-mail in error, please notify us by reply e-mail. Thank you for your cooperation.



Adrienne Rockenhaus <adrienne@rockenhaus.com>

NOTICE OF EVIDENCE: Documented History of Sexual Assault and Facilitated Violence

Adrienne Rockenhaus <adrienne@rockenhaus.com>

Thu, Feb 12, 2026 at 11:14 AM

To: Kaycee Berente <kaycee_berente@fd.org>

Bcc: Adrienne Rockenhaus <adrienne@rockenhaus.com>

Kaycee,

To be absolutely clear about the divorce and PPO I am filing at 8:00 AM tomorrow: do not mistake my intent for a bluff. I am prepared to enter the following into the public record to ensure you, your office, and the BOP is held liable for every second of their "Safe Release" malpractice:

- * Sexual Coercion: I discovered and possess the digital receipts for Viagra Conrad purchased and used to facilitate the physical and sexual coercion of me prior to his 2025 arrest, after I explicitly withdrew consent.

- * 2023 Physical Assault: I hold the police report from 2023 documenting his physical assault against me.

- * Threat Evidence: I hold numerous recordings of Conrad over the years threatening my life and safety.

- * Facilitation of Violence: I discovered and hold the proof that Conrad lied about securing legal counsel to protect me following my Sept 2024 assault by William Keene. He knowingly left me in a home with a violent aggressor while lying about "keeping me safe".

If you or the BOP proceed with a plan that involves Conrad having access to me, my home, or my sanctuary, before he receives critical stabilizing medical care at a hospital or VA facility immediately upon release, you are doing so with full knowledge that you are releasing a medically unstable abuser to his victim.

This is the end of the conversation. I am filing the divorce, the Fee Waiver, and the PPO at 8:00 AM tomorrow. Any response from you that does not include a direct medical transfer to a VA Medical Center will be treated as an admission of Deliberate Indifference and criminal complicity.

By facilitating Conrad's 7:05 AM harassment strike using a 20-year-old "side piece" while ignoring a Care Level 4 medical crisis, you have officially abandoned any pretense of professional ethics or law.

Adrienne Rockenhaus



Adrienne Rockenhaus <adrienne@cannabytes.net>

Person in Custody: ROCKENHAUS, CONRAD

'CorrLinks' via info@cannabytes.net <info@cannabytes.net>

Thu, Feb 12, 2026 at 7:05 AM

Reply-To: CorrLinks <info@corrlinks.com>

To: brenewmang@cannabytes.net

This is a system generated message informing you that the above-named person is a federal person in custody who seeks to add you to his/her contact list for exchanging electronic messages. There is no message from the person in custody at this time.

You can ACCEPT this person in custody's request or BLOCK this individual or all federal persons in custody from contacting you via electronic messaging at [Corrlinks.com](https://www.bop.gov/inmates/trulincs.jsp). To register with CorrLinks you must enter the email address that received this notice along with the identification code below.

Email Address: brenewmang@cannabytes.net

Identification Code: NMD17CP7

This identification code will expire in 10 days.

By approving electronic correspondence with federal persons in custody, you consent to have the Bureau of Prisons staff monitor the content of all electronic messages exchanged.

Once you have registered with CorrLinks and approved the person in custody for correspondence, the person in custody will be notified electronically.

For additional information related to this program, please visit the <https://www.bop.gov/inmates/trulincs.jsp> FAQ page.

Este es un mensaje generado por el sistema que le informa que la persona mencionada es un persona bajo custodia federal que pretende añadirlo a usted a su lista de contactos para intercambiar mensajes electrónicos. No hay ningún mensaje del persona bajo custodia en este momento.

Usted puede ACEPTAR esta petición del persona bajo custodia o BLOQUEAR a esta persona o a todos los personas bajo custodia federales de contactarlo a usted a través de la mensajería electrónica en [Corrlinks.com](https://www.bop.gov/inmates/trulincs.jsp). Para inscribirse en CorrLinks debe introducir la dirección de correo electrónico que recibió esta notificación, junto con el código de identificación a continuación.

Dirección de correo electrónico: brenewmang@cannabytes.net

Código de identificación: NMD17CP7

Este código de identificación expirará en 10 días.

Al aprobar la correspondencia electrónica con personas bajo custodia federales usted esta consintiendo a que personal de la Oficina de Prisiones supervise el contenido informativo de todos los mensajes electrónicos intercambiados y cumplir con todas las reglas y procedimientos del Programa.

Una vez registrado en Corrlinks y aprobado para la correspondencia el persona bajo custodia será notificado por vía electrónica.

Para obtener información adicional relacionada con este programa, por favor visite la página de preguntas frecuentes <https://www.bop.gov/inmates/trulincs.jsp>.

To unsubscribe click [here](#).

Para anular su suscripción, haga clic [aquí](#)



Adrienne Rockenhaus <adrienne@cannabytes.net>

Person in Custody: ROCKENHAUS, CONRAD

'CorrLinks' via info@cannabytes.net <info@cannabytes.net>

Sat, Feb 14, 2026 at 4:05 PM

Reply-To: CorrLinks <info@corrlinks.com>

To: sevbivens@cannabytes.net

This is a system generated message informing you that the above-named person is a federal person in custody who seeks to add you to his/her contact list for exchanging electronic messages. There is no message from the person in custody at this time.

You can ACCEPT this person in custody's request or BLOCK this individual or all federal persons in custody from contacting you via electronic messaging at [Corrlinks.com](https://www.bop.gov/inmates/trulincs.jsp). To register with CorrLinks you must enter the email address that received this notice along with the identification code below.

Email Address: sevbivens@cannabytes.net

Identification Code: XJR188WW

This identification code will expire in 10 days.

By approving electronic correspondence with federal persons in custody, you consent to have the Bureau of Prisons staff monitor the content of all electronic messages exchanged.

Once you have registered with CorrLinks and approved the person in custody for correspondence, the person in custody will be notified electronically.

For additional information related to this program, please visit the <https://www.bop.gov/inmates/trulincs.jsp> FAQ page.

Este es un mensaje generado por el sistema que le informa que la persona mencionada es un persona bajo custodia federal que pretende añadirlo a usted a su lista de contactos para intercambiar mensajes electrónicos. No hay ningún mensaje del persona bajo custodia en este momento.

Usted puede ACEPTAR esta petición del persona bajo custodia o BLOQUEAR a esta persona o a todos los personas bajo custodia federales de contactarlo a usted a través de la mensajería electrónica en [Corrlinks.com](https://www.bop.gov/inmates/trulincs.jsp). Para inscribirse en CorrLinks debe introducir la dirección de correo electrónico que recibió esta notificación, junto con el código de identificación a continuación.

Dirección de correo electrónico: sevbivens@cannabytes.net

Código de identificación: XJR188WW

Este código de identificación expirará en 10 días.

Al aprobar la correspondencia electrónica con personas bajo custodia federales usted esta consintiendo a que personal de la Oficina de Prisiones supervise el contenido informativo de todos los mensajes electrónicos intercambiados y cumplir con todas las reglas y procedimientos del Programa.

Una vez registrado en Corrlinks y aprobado para la correspondencia el persona bajo custodia será notificado por vía electrónica.

Para obtener información adicional relacionada con este programa, por favor visite la página de preguntas frecuentes <https://www.bop.gov/inmates/trulincs.jsp>.

To unsubscribe click [here](#).

Para anular su suscripción, haga clic [aquí](#)

EXHIBIT A: EVIDENCE OF CHEMICAL PREMEDITATION, MEDICAL DECEPTION, AND SEXUAL COERCION

To the Honorable Judge:

The attached digital receipts document the Respondent's calculated medical deception and premeditated stockpiling of maximum-strength erectile dysfunction medication (100mg Sildenafil).

1. The "Double-Fill" Deception: Records prove that the Respondent was actively "double filling" prescriptions to bypass federal safety limits and hoard medication. On June 3, 2024, he obtained a private prescription for Sildenafil through a telehealth app ("Dr. B"). Just four days later, on June 7, 2024, he obtained a duplicate supply from the Department of Veterans Affairs (VA), deliberately hiding his private supply from his federal doctors. He continued to maintain this private supply line into 2025.

2. Chemical Premeditation and Sexual Coercion: He utilized these secretly stockpiled medications to chemically override his body's natural limits and forcefully facilitate physical and sexual coercion against me. He executed this chemical coercion while I was medically incapacitated and explicitly revoking consent due to severe trauma.

Conclusion: This calculated medical deception demonstrates a complete lack of impulse control and a predatory willingness to manipulate pharmaceutical supply lines to violate my physical boundaries, leaving me in extreme, paralyzing fear for my safety upon his release.

Dr. B Support Team

1 message

Dr. B Support Team <hi@hidrb.com>
Reply-To: "Dr. B Support Team" <hi@hidrb.com>
To: Conrad Rockenhaus <conrad@rockenhaus.com>

Mon, Feb 3, 2025 at 4:25 PM

Your request (103812) has been updated. To add additional comments, reply to this email.

**Argeline (Dr. B)**

Feb 3, 2025, 4:25PM EST

Hello Conrad,

Thanks for reaching out to Dr. B.

We checked this for you and left a note to your provider that you are requesting a refill of your Sildenafil 100 mg tablet. You should receive an email confirmation once this is approved.

If you need anything at all, you can always reach out to us and we'll be happy to help.

Sincerely,

Argeline

Dr. B Support Team

For further inquiries, respond to this email or visit hidrb.com

**Conrad Rockenhaus**

Feb 3, 2025, 3:57PM EST

Hello,

I renewed but it wouldn't let me select 100mg viagra like I have filled....
Sent from my iPhone

On Dec 17, 2024, at 8:11 AM, Dr. B <no-reply@mail.hidrb.com> wrote:

Hi Conrad,

This is Nicole Baldwin NP. I wanted to let you know it's almost time for your prescription refill. We're here to help make the process convenient and hassle-free.

Go to Dr. B to update your health assessment and, if medically appropriate, a provider will renew your prescription and send it over to your pharmacy of choice.

Renew Your Prescription

If you have any questions about your treatment plan, go to your [patient dashboard](#) for support.

Take care,

Nicole Baldwin NP

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[2980 McFarlane Rd, Suite 200, Miami, FL 33133](#)

[Privacy Policy](#)
[Terms and Conditions](#)

[MRKG76-V4DKK]

Request #103812: How would you rate the support you received?

1 message

Dr. B Support Team <hi@hidrb.com>
Reply-To: "Dr. B Support Team" <hi@hidrb.com>
To: Conrad Rockenhaus <conrad@rockenhaus.com>

Mon, Feb 3, 2025 at 6:01 PM

Hello Conrad Rockenhaus,

We hope you're having a great day!

You recently contacted our support team, and we'd love to hear your thoughts about your experience. We'd be grateful if you could take a few minutes to rate our service. We kept it short since we know how valuable your time is.

How would you rate your experience with our Customer Service?

Bad



Okay



Good

Here's a reminder of what this request was about:

**Argeline (Dr. B)**

Feb 3, 2025, 4:25PM EST

Hello Conrad,

Thanks for reaching out to Dr. B.

We checked this for you and left a note to your provider that you are requesting a refill of your Sildenafil 100 mg tablet. You should receive an email confirmation once this is approved.

If you need anything at all, you can always reach out to us and we'll be happy to help.

Sincerely,

Argeline

Dr. B Support Team

For further inquiries, respond to this email or visit hidrb.com



Conrad Rockenhaus

Feb 3, 2025, 3:57PM EST

Hello,

I renewed but it wouldn't let me select 100mg viagra like I have filled....

Sent from my iPhone

On Dec 17, 2024, at 8:11 AM, Dr. B <no-reply@mail.hidrb.com> wrote:

Hi Conrad,

This is Nicole Baldwin NP. I wanted to let you know it's almost time for your prescription refill. We're here to help make the process convenient and hassle-free.

Go to Dr. B to update your health assessment and, if medically appropriate, a provider will renew your prescription and send it over to your pharmacy of choice.

Renew Your Prescription

If you have any questions about your treatment plan, go to your [patient dashboard](#) for support.

Take care,

Nicole Baldwin NP

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[2980 McFarlane Rd, Suite 200, Miami, FL 33133](#)

[MRKG76-V4DKK]



Conrad Rockenhaus <conrad@rockenhaus.com>

Dr. B Support Team

1 message

Dr. B <support@hidrb.zendesk.com>
Reply-To: "Dr. B" <support+id24NWM3-R9KKV@hidrb.zendesk.com>
To: Conrad Rockenhaus <conrad@rockenhaus.com>

Thu, Apr 17, 2025 at 2:04 PM

Your request (117133) has been updated. To add additional comments, reply to this email.



Argeline (Dr. B)

Apr 17, 2025, 2:04PM EDT

Hello Conrad,

Thanks for reaching out to Dr. B.

We left a note in your consultation that you are requesting refill for sildenafil 100mg. Please make sure to select refill consultation when renewing your prescription and have a photo of your pill bottle ready. You should receive an email once your consultation is approved and the prescription is sent to your selected pharmacy.

If you need anything at all, you can always reach out to us and we'll be happy to help.

Sincerely,

Argeline

Dr. B Support Team

For further inquiries, respond to this email or visit hidrb.com

[24NWM3-R9KKV]

Request #117133: How would you rate the support you received?

Dr. B <support@hidrb.zendesk.com>

Thu, Apr 17, 2025 at 4:02 PM

Reply-To: "Dr. B" <support+id24NWM3-R9KKV@hidrb.zendesk.com>

To: Conrad Rockenhaus <conrad@rockenhaus.com>

Hello Conrad Rockenhaus,

We hope you're having a great day!

You recently contacted our support team, and we'd love to hear your thoughts about your experience. We'd be grateful if you could take a few minutes to rate our service. We kept it short since we know how valuable your time is.

How would you rate your experience with our Customer Service?

Bad



Okay



Good

Here's a reminder of what this request was about:

**Argeline (Dr. B)**

Apr 17, 2025, 2:04PM EDT

Hello Conrad,

Thanks for reaching out to Dr. B.

We left a note in your consultation that you are requesting refill for sildenafil 100mg. Please make sure to select refill consultation when renewing your prescription and have a photo of your pill bottle ready. You should receive an email once your consultation is approved and the prescription is sent to your selected pharmacy.

If you need anything at all, you can always reach out to us and we'll be happy to help.

Sincerely,
Argeline

Dr. B Support Team

For further inquiries, respond to this email or visit hidrb.com

[24NWM3-R9KKV]



Conrad Rockenhaus <conrad@rockenhaus.com>

Dr. B Support Team

2 messages

Dr. B <support@hidrb.zendesk.com>
Reply-To: "Dr. B" <support+id72856@hidrb.zendesk.com>
To: Conrad Rockenhaus <conrad@rockenhaus.com>

Mon, Jun 3, 2024 at 9:17 AM

Your request (72856) has been updated. To add additional comments, reply to this email.



Dimple (Dr. B)

Jun 3, 2024, 9:17AM EDT

Hello Conrad,

Thanks for reaching out to Dr. B.

We are sorry to hear about that. We've added a note to your consultation indicating that the medication refill should remain sildenafil 100mg. You'll receive an email notification once your consultation is approved.

If you need anything at all, you can always reach out to us and we'll be happy to help.

Sincerely,

Argeline

Dr. B Support Team

For further inquiries, respond to this email or visit hidrb.com

[2RM2X6-YNJLV]

Dr. B <support@hidrb.zendesk.com>
Reply-To: "Dr. B" <support+id72856@hidrb.zendesk.com>
To: Conrad Rockenhaus <conrad@rockenhaus.com>

Mon, Jun 3, 2024 at 4:03 PM

Your request (72856) has been updated. To add additional comments, reply to this email.

00018



Dimple (Dr. B)

Jun 3, 2024, 4:03PM EDT

Hello Conrad,

We're glad to inform you that a medical provider has approved your prescription for Sildenafil 100 mg tablet. An email confirming all the details has been sent to you. If you don't find the email in your inbox, please check your spam or junk folder in case it was mistakenly sent there. You can call the pharmacy after 30 minutes to confirm pick-up time. Below again are the pharmacy details for your reference:

MEIJER PHARMACY #054

20401 HAGGERTY RD, NORTHVILLE MI 48167

Phone: (248) 449-5733

If you need anything at all, you can always reach out to us and we'll be happy to help.

Sincerely,

Argeline

Dr. B Support Team

For further inquiries, respond to this email or visit hidrb.com



Dimple (Dr. B)

Jun 3, 2024, 9:17AM EDT

Hello Conrad,

Thanks for reaching out to Dr. B.

We are sorry to hear about that. We've added a note to your consultation indicating that the medication refill should remain sildenafil 100mg. You'll receive an email notification once your consultation is approved.

If you need anything at all, you can always reach out to us and we'll be happy to help.

Sincerely,

Argeline

Dr. B Support Team

For further inquiries, respond to this email or visit hidrb.com

[2RM2X6-YNJLV]

Request #72856: How would you rate the support you received?

Dr. B <support@hidrb.zendesk.com>
Reply-To: "Dr. B" <support+id72856@hidrb.zendesk.com>
To: Conrad Rockenhaus <conrad@rockenhaus.com>

Mon, Jun 3, 2024 at 6:01 PM

Hello Conrad Rockenhaus,

We hope you're having a great day!

You recently contacted our support team, and we'd love to hear your thoughts about your experience. We'd be grateful if you could take a few minutes to rate our service. We kept it short since we know how valuable your time is.

How would you rate your experience with our Customer Service?



Here's a reminder of what this request was about:

**Dimple (Dr. B)**

Jun 3, 2024, 4:03PM EDT

Hello Conrad,

We're glad to inform you that a medical provider has approved your prescription for Sildenafil 100 mg tablet. An email confirming all the details has been sent to you. If you don't find the email in your inbox, please check your spam or junk folder in case it was mistakenly sent there. You can call the pharmacy after 30 minutes to confirm pick-up time. Below again are the pharmacy details for your reference:

MEIJER PHARMACY #054[20401 HAGGERTY RD, NORTHVILLE MI 48167](#)

Phone: (248) 449-5733

If you need anything at all, you can always reach out to us and we'll be happy to help.

Sincerely,
Argeline
Dr. B Support Team

For further inquiries, respond to this email or visit hidrb.com



Dimple (Dr. B)

Jun 3, 2024, 9:17AM EDT

Hello Conrad,

Thanks for reaching out to Dr. B.

We are sorry to hear about that. We've added a note to your consultation indicating that the medication refill should remain sildenafil 100mg. You'll receive an email notification once your consultation is approved.

If you need anything at all, you can always reach out to us and we'll be happy to help.

Sincerely,
Argeline
Dr. B Support Team

For further inquiries, respond to this email or visit hidrb.com

[2RM2X6-YNJLV]

EXHIBIT C: EVIDENCE OF COERCIVE CONTROL, LITIGATION HARASSMENT, AND WITNESS SILENCING

To the Honorable Judge:

The attached documents demonstrate how the Respondent is actively weaponizing the federal court docket to psychologically terrorize me, silence my reports of his sexual abuse, and lay a trap for his impending release on March 2, 2026. He is utilizing litigation harassment to assert coercive control through contradictory actions:

1. The Private Demand: The first document is an email from his defense attorney (dated Feb 9, 2026) relaying the Respondent's direct instructions demanding I pick him up at the prison gates so we can "go home."

2. The Public Legal Trap: The second document is a handwritten motion he filed in federal court (docketed Feb 18). In it, he swears to a judge that he "revoked" my legal standing weeks prior on Jan 25.

3. Retaliatory Witness Silencing: As demonstrated in the prior exhibit (**Exhibit B**), I formally notified the Respondent's attorney on February 12, 2026, of his history of sexual coercion and physical assault. In direct retaliation, the Respondent is using this federal court motion to explicitly demand the judge "no longer accept any filings" from me. He is using the court docket to legally gag a domestic violence victim and strip away my protections just days before he is released.

Conclusion: The Respondent cannot legally tell a federal judge that he stripped me of all authority in January, while simultaneously ordering me to be his release custodian in February. He is attempting to legally isolate me, discredit my reports of his sexual predation, and privately force his way into my home upon release. This litigation harassment places me in extreme fear for my safety and liberty.



Adrienne Rockenhaus <adrienne@rockenhaus.com>

Message from Conrad re: Release

Kaycee Berente <Kaycee_Berente@fd.org>
To: "adrienne@rockenhaus.com" <adrienne@rockenhaus.com>

Mon, Feb 9, 2026 at 10:47 AM

Hi Adrienne,

Conrad wanted me to pass along the following:

"Please convey to Adrienne that everything is going well with my release planning and that all she needs to do is pick me up. I have two dedicated VA Social Workers that are working with me on my case, they know that I have an untreated head trauma from my arrest on September 7th and are making arrangements for me to see Neurology to start being treated. They also know I have additional health issues that they have to treat me for. Arrangements have been made, things are good, I really just need her to pick me up and we'll be good to go. I will give her all of the information once she picks me up, the staff here at the BOP aren't going to be too helpful. The bottom line is, all of the arrangements for my follow-on care have been made, my medical care for my head trauma and other issues has been lined up and appointments have been made, I really just need her to focus on making sure she's safely here on March 2nd at 8 AM so she can meet me at the door of the FDC so we can go home."

Best,

Kaycee Berente

Assistant Federal Public Defender

Appellate Unit

Southern District of Ohio

10 W Broad St, Suite 1020

Columbus, OH 43215

Office: 614-469-2999

Direct: 614-469-4130

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00024

notified that any dissemination or copying of this e-mail is strictly prohibited. If you have received this e-mail in error, please notify us by reply e-mail. Thank you for your cooperation.

United States District Court
Eastern District of Michigan
Southern Division

FILED
FEB 18 2026
CLERK'S OFFICE
DETROIT

Conrad Rockenhau
Petitioner

v.

Tarnesia Pringle
Respondent

Case No 25-cv-12913
Hon. Jonathan J.C. Grey

MOTION

Adrienne Rockenhau filed a Habeas Corpus Petition as a Next Friend and has since appealed the decision of this Court. I implore this Court to no longer accept any filings from Adrienne Rockenhau as my Next Friend and Power of Attorney.

Please be advised that the Power of Attorney that I granted to her was revoked by me on 1/25/26

Respectfully Submitted,

Conrad Rockenhau

2/11/2026

Conrad Beckenhous

NAME

39400460

REG. NO.

FEDERAL CORRECTIONAL INSTITUTION
P.O. BOX 1000
MILAN, MICHIGAN 48160

METROPLEX MI 480

12 FEB 2026 PM 9 L



U.S. MARSHALS

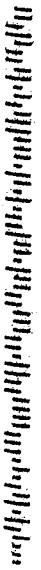
Theodore Levin United States Courthouse
Attn: Court Clerk

231 West Lafayette Boulevard, 5th Floor
Detroit, MI 48226

RECEIVED
FEB 18 2026

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U.S. DISTRICT COURT

48226-277758



UNITED STATES DISTRICT COURT
EASTERN DISTRICT OF MICHIGAN
SOUTHERN DIVISION

FILED
FEB 18 2026

CLERK'S OFFICE
DETROIT

Adrienne Rockenhans
Conrad Rockenhans
Plaintiffs

v
Dean Thomas
Tefry Kunal
Stychnos Agipu, et al
Defendants

Case No
2:25-cv-12716-JMB-SJB

MOTION TO SEVER

Plaintiff Conrad Rockenhans has been advised that his Bivens claim has been combined with his wife, Plaintiff Adrienne Rockenhans. Due to various factors, including a breakdown of the interpersonal familial framework, Plaintiff Conrad Rockenhans believes that combining his claims with his spouse's claims will place his claim in jeopardy and brings prejudice to his case.

Plaintiff Conrad Rockenhans requests if additional briefing and/or evidence is required to grant this motion, he be allowed to submit it under seal.

Respectfully Submitted,

Conrad Rockenhans
2/12/2026

Conrad Beckhaus

NAME

34400-460

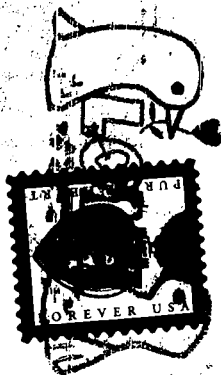
REG. NO.

FEDERAL CORRECTIONAL INSTITUTION
P.O. BOX 1000
MILAN, MICHIGAN 48160

RECEIVED
FEB 18 2026

CLERK'S OFFICE
U.S. DISTRICT COURT

METROPLEX MI 480
13 FEB 2026 PM 6 L



*United States District Court
Eastern District of Michigan
Attn: Court Clerk
231 West Lafayette Boulevard, SM F1
Detroit, MI 48226*

48226-270099

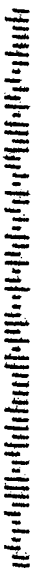


EXHIBIT D: MEDICAL EVIDENCE OF PRIOR VIOLENCE AND POLICE INTERVENTION

To the Honorable Judge:

Because I am filing this petition as an immediate *ex parte* emergency to secure my physical safety prior to the Respondent's March 2nd release, I have not attached the formal Plymouth Police Department incident report from May 4, 2023.

Instead, I am attaching the Respondent's VA Emergency Department medical records from that exact week for transparency.

Because this is a lengthy medical file, **I direct the Court's attention specifically to the clinical notes dated May 4, 2023**, which serve as absolute proof that law enforcement responded to our residence for domestic violence and validate my current fear for my life:

1. Proof of the Police Ultimatum for Aggression In the Mental Health Consult note dated May 4, 2023 (authored by Nancy Bigelow, NP), the medical record explicitly documents the police intervention, stating:

"Police were called to the residence and Pt reports they gave him a choice of going to jail or coming to the ED."

2. Proof of the Domestic Altercation In the Emergency Department Flowsheet dated May 4, 2023 (Authored by Brandon Templeton, RN), the triage note explicitly states:

"Patient states he was in an altercation with wife last night that spilled over into today when PD were called."

The Respondent chose the hospital to avoid arrest. This establishes a documented, medical baseline of physical violence that proves my physical safety is in imminent danger upon his release.

LOCAL TITLE: EMERGENCY DEPARTMENT MENTAL HEALTH CONSULT
STANDARD TITLE: EMERGENCY DEPT NOTE
DATE OF NOTE: MAY 04, 2023@15:41 ENTRY DATE: MAY 04, 2023@15:41:31
AUTHOR: BIGELOW,NANCY MARIE EXP COSIGNER:
URGENCY: STATUS: COMPLETED

*** EMERGENCY DEPARTMENT MENTAL HEALTH CONSULT Has ADDENDA ***

EMERGENCY DEPT MENTAL HEALTH NOTE

PATIENT IDENTIFYING DATA

Name: ROCKENHAUS,CONRAD ALAN

Age: 41

Gender: MALE

Ethnicity: WHITE

Marital status: DIVORCED

Total time: 90 min including F2F with Pt and care coordination.
Pt was interviewed in the ED together with Nancy Bigelow, NP.

DETAILS OF PRESENTING PROBLEM:

Pt is a 41 y/o male with PPHx including Opioid use d/o on MAT, Bipolar d/o, PTSD, anxiety and depression with multiple past suicide attempts and psychiatric hospitalizations (most recently on AIMH April 26, 2023 to May 1, 2023), who presented to the ED via EMS after an alleged argument with his fiancée. Police were called to the residence and Pt reports they gave him a choice of going to jail or coming to the ED.

On interview today he denies any mood disturbance or thoughts of self-harm, suicide, or thoughts of harming others. Pt endorses stable mood since discharge from AIMH on 5-1-2023. He reports more stressors in his relationship since starting couples therapy on 5-2-2023, stating "I feel like she doesn't validate my stressors . . .and she doesn't like when I point this out to the therapist".

The alleged altercation happened today when he says he had a seizure and his fiancée got upset and started "yelling at me" and that "I can't be your caretaker" because she could lose her job. The fiancée then insisted he lie down after his "seizure", and when he awakened, she called him "pathetic" and "a limp-dick man" and wanted him to leave. The Pt reports he then said "I'm not leaving because I pay the rent".

At that point the fiancée called 911, stating that she told the police that he pushed her which Pt denies. Pt then stated that he also called 911 "so they could hear my side of the story". When the police arrived Pt states the police believed that he did not push the fiancée but gave him the ultimatum of "going to jail or going to the hospital to appease her". Reports the police told him he could return back to the residence after being evaluated.

MH team informed the Pt that we will need to verify with the police department that he may return home or what the appropriate steps will be from ED. Pt is agreeable and states he lives in Plymouth, MI and the Plymouth Police Department was called. The Police department verified they have no concerns for him to be discharged and return to his residence.

Denies any issues with sleep or appetite. Reports compliance with medication regimen and intention to attend post discharge appointments; Pt also reports he is planning on attending all f/u appointments including appointment tomorrow.

PSYCHIATRIC HISTORY:

- Per chart, Dx include Bipolar I (moods & over-spending), opioid use d/o related to chronic back pain (now on suboxone), PTSD, depression, anxiety, possible delusional d/o with paranoid and sometimes grandiose delusions.
- Previous treatment in TX, recently established care in SUD-C here.
- Multiple past psych admissions (at least 12), most recently on AIMH from 4-26-2023 to 5-1-2023.
- Multiple past psych med trials including Ambien, Klonopin, Valium, Ativan, Eszopiclone, Zoloft, Remeron, Effexor, Cymbalta, Doxepin, Nortriptyline, Seroquel, Haldol, Risperdal consta, Geodon, Depakote, Lithium, Buspar, Hydroxyzine, Propranolol, Prazosin, Trazodone, Suboxone.
- Per chart, multiple suicide attempts. Pt discussed today's attempt via lying on railroad tracks, fiance found him by tracking his location on his phone then brought him to the ED. Pt reported the most recent attempt prior to today was in Oct 2022 - ingested ~50 Metformin tablets in the context of difficulty adjusting to civilian life after incarceration, argument w/ fianc?e and opiate use.

SUBSTANCE USE HISTORY:

- OUD on MAT with buprenorphine
- denies cannabis use "I quit" since discharging from AIMH unit on 5-1-2023
- denies etoh, denies illicit drug use
- occasional adderall from a friend who has a prescription

PSYCHOSOCIAL HISTORY AND STRESSORS:

Divorced, 3 children (notes from 4-27-2023 indicated Pt has 2 children). Recently moved from TX to MI and lives with his fiance, they've been together x3 years. Current income from 100% VA SC. Per chart, Pt has a BA in computer science, past employment- eb/security/computer consulting business. Currently on supervised release for 3 years after ~40 months of incarceration for computer hacking. Pt cites his fiance and his mother as main social supports. Independent w/ ADLs. Denied access to firearms.

Military Service - Navy 2003-2015

Percent SC: 100

Rated disabilities:

SCARS 0% SC

BIPOLAR DISORDER 70% SC

TRAUMATIC BRAIN DISEASE 10% SC

LIMITED FLEXION OF THIGH 0% SC

LIMITED MOTION OF ARM 0% SC

HYPERTENSIVE VASCULAR DISEASE 0% SC

SCARS 0% SC

SCARS 0% SC

LIMITED MOTION OF ARM 0% SC

SLEEP APNEA SYNDROMES 50% SC

MIGRAINE HEADACHES 30% SC

LIMITED FLEXION OF THIGH 10% SC

SCARS 0% SC
LUMBOSACRAL OR CERVICAL STRAIN 0% SC
PARALYSIS OF SCIATIC NERVE 10% SC
LUMBOSACRAL OR CERVICAL STRAIN 10% SC
IRRITABLE COLON 0% SC
LIMITED FLEXION OF KNEE 10% SC
TINNITUS 10% SC
SEIZURE DISORDER 100% SC

MEDICAL HISTORY:

Per chart:

- Seizure disorder
- Sleep apnea, not using CPAP ("ex-wife stole it")
- Migraine
- Sciatica
- Chronic back pain
- Tendinitis
- Hypertension
- Neck pain
- Atopic dermatitis
- Hearing loss
- Morbid obesity
- Diabetes
- Allergic rhinitis
- Pulmonary vein injury
- GERD

Active Outpatient Medications (including Supplies):

-
- 1) ACETAMINOPHEN 250/ASA 250/CAFF 65MG TAB TAKE 2 ACTIVE
TABLETS BY MOUTH EVERY DAY AS NEEDED FOR HEADACHE
 - 2) ACETAMINOPHEN 500MG TAB TAKE 1 TABLET BY MOUTH TWICE ACTIVE
A DAY AS NEEDED FOR PAIN
 - 3) BUPRENORPHINE 8MG/NALOXONE 2MG SL TAB DISSOLVE 1 ACTIVE
TABLET UNDER THE TONGUE EVERY MORNING - THIS RX FOR
5-1-23 TO 5-16-23
 - 4) FAMOTIDINE 20MG TAB TAKE ONE TABLET BY MOUTH TWICE A ACTIVE
DAY FOR HEARTBURN
 - 5) FOLIC ACID 1MG TAB TAKE ONE TABLET BY MOUTH ONCE ACTIVE
DAILY
 - 6) LITHIUM CARBONATE 300MG CAP TAKE FOUR CAPSULES BY ACTIVE
MOUTH AT BEDTIME FOR MENTAL HEALTH
 - 7) PILL SPLITTER-EA USE SPLITTER -- AS DIRECTED TO SPLIT ACTIVE
TABLETS
 - 8) PRAZOSIN HCL 1MG CAP TAKE THREE CAPSULES BY MOUTH ACTIVE
TWICE A DAY FOR NIGHTMARES OR SLEEP DISRUPTION
 - 9) RISPERIDONE 1MG TAB TAKE ONE-HALF TABLET BY MOUTH ACTIVE
THREE TIMES A DAY AS NEEDED FOR ANXIETY AND
DIFFICULTY SLEEPING
 - 10) TRAZODONE HCL 100MG TAB TAKE ONE TABLET BY MOUTH AT ACTIVE
BEDTIME FOR SLEEP
 - 11) VITAMIN B COMPLEX/VITAMIN C CAP/TAB TAKE 1 TABLET BY ACTIVE
MOUTH ONCE DAILY FOR VITAMIN SUPPLEMENT

MENTAL STATUS EXAMINATION:

Alert: sitting up in bed in VA attire good hygiene, good eye contact, pleasant and cooperative in interview

Orientation: A & O x 4

Psychomotor: no psychomotor agitation/retardation. No tremors or tics noted.

Speech: normal rate, rhythm, loud volume x times

Mood: "fine"

Affect: broad, reactive; mood congruent

Thought processes: overall logical, linear, goal-directed

Concentration: appropriate to interview; no deficits noted

Thought content: Denies SI/HI intent or plan. Denies SIB thoughts/urges.

Hallucinations: Denies AVH. No evidence of responding to internal stimuli or thought blocking.

Delusions: No paranoid or delusional statements elicited or expressed.

Insight: limited-to-fair

Judgement: limited-to-fair

SUICIDE RISK:

See CSRE

HOMICIDE/VIOLENCE RISK: No. Plymouth Police Department contacted by MH team to verify they have no safety concerns to discharge Pt to return to the residence; Plymouth PD have no concerns for Pt to be discharged.

DSM V DIAGNOSES:

- mood disorder unspecified d/o

- bipolar d/o by history

- OUD severe on MAT with buprenorphine

- chronic PTSD by history

- Delusional d/o by history

- cannabis use daily (recently quit)

IMPRESSION:

Pt is a 41 y/o male with psychiatric history of bipolar d/o, chronic PTSD, delusional d/o, OUD severe on MAT, past suicide attempts (most recent report of S.A is lying on railroad tracks on 4-26-2023), numerous inpatient psychiatric admissions (most recent AIMH 4-26-2023 to 5-1-2023), has received MH treatment at the inpatient and outpatient levels of care who presents to AAVAED via EMS for purported SI after argument with fiance.

Pt denies low mood, SI, or HI now or at any time since discharge from AIMH unit on 5-1-2023 and reports that the police gave him the choice of going to jail or going to get evaluated in the ED after 911 was called after argument with fiance. Pt chose to come to ED to be evaluated and "appease" his fiance. The Plymouth Police Dept. was contacted by the ED MH team and report each party said they pushed the other and they also confirm there are no safety concerns for the Pt to return to residence.

Pt provided his version of events that led to ED presentation and in summary: he and fiance had argument today after he had a "seizure" and she said she could not be his caretaker. The argument escalated to the fiance verbally

insulting him and telling him to leave; the Pt said he will not leave bk he pays the rent. Each party called 911 and the police essentially sent the Pt to the ED to separate them for a cooling off period.

Pt is pleasant t/o the interview with all ED staff. Pt presentation appears to chronically dramatic/grandiose which may be consistent with mania, emotional distress, personality d/o, mood d/o, and/or delusional disorder and may be obfuscated by OUD pathology. He was recently observed on AIMH unit x 4 days, and while he was primarily admitted for a depressive episode, suspicion was maintained for a personality contribution. This was reinforced by his rapid improvement without specific intervention. He was maintained on his home lithium, prazosin, and trazodone. Additionally low-dose risperidone was added to target depression, anxiety, and insomnia. Suboxone dosing was not altered during the course of his stay.

Pt does not meet criteria for involuntary admission. Due to lack of acute psychiatric symptoms, available support, independence with ADLs, willingness to seek help, future-orientation, engagement with outpatient psychiatry for medication management and no safety concerns will discharge Pt to home with outpatient f/u already in place s/p discharge from AIMH on 5-1-2023

PLAN:

- Patient is safe for d/c to home with outpatient follow up.
- F/u at his next scheduled outpatient appointment 5-5-2023
- Reviewed Safety Plan and provided with copy.
- Counseling provided re: alternative coping strategies, attending f/u MH appointments and taking medication as prescribed for best outcomes for MH recovery and maintenance.
- Provided Veteran's Crisis Line #. Pt educated re worsening s/sx and advised to call if these occur.
- Pt verbalized understanding and agreement with plan.

This case discussed with ED attending Dr. Cutter and Alisa Meloche LMSW

/es/ NANCY MARIE BIGELOW

Nurse Practitioner, Psychiatry

Signed: 05/04/2023 16:19

Receipt Acknowledged By:

05/05/2023 07:59 /es/ JESICA LEIGH KALMBACH
Staff Psychologist

05/04/2023 16:25 /es/ ALISA J MELOCHE LMSW
Psychiatric Social Worker

05/08/2023 09:38 /es/ MARC A. OLSON, MD
Staff Physician, Substance Abuse Clinic

05/04/2023 ADDENDUM

STATUS: COMPLETED

Writer called back by Emergency Department Physician regarding the care of Mr. Rockenhaus. Previously seen by ER Mental Health Consult team (see notes above), and cleared for d/c. Patient Mr. Rockenhaus reportedly called fiance (Adrienne Blair 734-585-6277) to tell her he was coming home. Ms. Blair called into the AAVA ED to report that she had safety concerns (text messages saying that he was going to plan on harming himself if SHE had to leave house to go to the hospital

for her own mental health care). Also reported suicide attempt of laying on tracks (of note, this was prior to last admission, and he has not had any suicidal attempts since most recent d/c from inpt admission on 5/1). Ms. Blair was hypervocal on the phone. Upon re-evaluation, pt continues to deny any SI/HI. He was offered voluntary admission to AIMH for further evaluation and management. Pt declined. Pt does not meet criteria for involuntary admission as he continues to deny symptoms.

Safety plan reviewed with patient. He has close follow-up with SUD-C Psychologist visit tomorrow 5/5 at 9am. Pt reported if he did have symptoms of SI/HI overnight he would discuss at visit tomorrow.

Originally safety planned with patient for him to go to a hotel for the night, so he would be able to make it to 9am appt at AAVA tomorrow morning, and so that he could allow some time for domestic argument to resolve. When writer left to print out safety plan, writer returned to patient talking to Ms. Blair on the phone, saying they had made up and she will pick him up. Writer again reinforced benefits of spending the night apart and recommended hotel for the evening. Writer offered patient 1mg Risperidone to help relax prior to d/c from ED.

/es/ MICHAEL JAMES FRANKLIN
Resident, Psychiatry
Signed: 05/04/2023 18:32

/es/ ANUSHA RANGANATHAN MD
Attending Physician, Psychiatry
Cosigned: 05/05/2023 10:02

SUICIDE RISK EVALUATION - COMPREHENSIVE

Details

Date: May 4, 2023
Location: ANN ARBOR VA MEDICAL CENTER
Written by: ALISA J MELOCHE
Signed by: ALISA J MELOCHE
Date signed: May 4, 2023

Notes

LOCAL TITLE: SUICIDE RISK EVALUATION - COMPREHENSIVE
STANDARD TITLE: SUICIDE PREVENTION RISK ASSESSMENT SCREENING NOT
DATE OF NOTE: MAY 04, 2023@15:32 ENTRY DATE: MAY 04, 2023@15:32:12
AUTHOR: MELOCHE,ALISA J EXP COSIGNER:
URGENCY: STATUS: COMPLETED

*** SUICIDE RISK EVALUATION - COMPREHENSIVE Has ADDENDA ***

Comprehensive Suicide Risk Evaluation

This is an update to an existing suicide risk evaluation.
The validity of the information contained within this evaluation is not in

question.

Suicidal Ideation

The most recent thoughts of engaging in suicide-related behavior were within the past 30 days.

Pt denies SI since he was hospitalized on AIMH 4/26/23.

The Veteran had suicidal intent at the time of the most recent ideation.

Description of intent: acting on SI on 4/26/23 by lying on railroad tracks. Denies SI, plan or intent since then.

The Veteran had a suicide plan at the time of the most recent ideation.

Describe: acting on SI on 4/26/23 by lying on railroad tracks. Denies SI, plan or intent since then.

The most recent suicidal ideation was the most severe ideation within the last 30 days.

The Veteran does not have access to lethal means (firearms)

The Veteran does not have access to other lethal means.

Suicidal Behavior

The Veteran has not made any suicide attempts since the last VA Comprehensive Suicide Risk Evaluation was completed.

The Veteran did not report any prior preparatory behaviors that have not been previously documented.

Warning Signs

The following warning signs are currently present for the Veteran:

None noted

Risk Factors

History of suicidal behavior(s)

Comment: multiple, most recent attempt 4/26/23

Recent psychosocial stressors

Please Describe: relationship problems

History of mental health hospitalization

Please Describe: multiple, most recently April 2023 on AIMH

Psychological conditions or symptoms

Please Describe: bipolar d/o, ptsd, anxiety, depression, opioid use d/o

Protective Factors and Reasons for Living

Access to and engagement with health care

Access to and engagement with mental health care

Reports motivation for mental health treatment

Has meaningful family relationships

Comment: mother

Hope for the future

Clinical Impressions:

The clinical impression of acute risk is Low ACUTE Risk.

As evidenced by: Denies SI, plan or intent.

The clinical impression of chronic risk is High CHRONIC Risk.
As evidenced by: Multiple past suicide attempts and psych admissions,
long MH/SUD history, limited coping skills, impulsivity, limited support
system.

Suicide Risk Mitigation Plan:

This treatment and care plan was developed in collaboration with the
Veteran.
This treatment and care plan was developed in collaboration with additional
healthcare providers.

List: Nancy Bigelow, NP

Risk Mitigation Plan:

Strategies for Managing Risk if the Veteran is Currently in the EMERGENCY
DEPARTMENT/URGENT CARE CENTER:

Suicide Prevention Coordinator was not alerted for consideration of a
Patient Record Flag Category I High Risk for Suicide.

Complete or update Veteran's safety plan

Inform Veteran that they will receive follow-up phone contact until
engaged in mental health care.

Obtain additional information from collateral sources

Comment: Plymouth PD

Provide Veteran with phone number for Veteran's Crisis Line: Dial 988
(Press 1), Text to 838255, or Chat

Re-evaluation:

Due to the dynamic nature of some warning signs, risk and protective factors,
suicide risk should be routinely re-evaluated. These risk management strategies
were chosen to address Veteran's current presentation and feasible treatment
options within the system of care. This plan should be re-evaluated over time.

/es/ ALISA J MELOCHE LMSW

Psychiatric Social Worker

Signed: 05/04/2023 15:46

05/05/2023 ADDENDUM STATUS: COMPLETED

Thank you for completing this CSRE and alerting and collaborating with
Suicide Prevention team. The Veteran currently has HRS PRF in chart.
Treatment providers should continue to assess for suicide risk during clinical
contacts. Suicide Prevention team will continue to monitor Veteran's care, per
HR PRF protocol. SP team appreciates the ongoing collaboration & coordination
of care by Veteran's treatment team.

/es/ MARY PAT TUCKER, LISW-S

Suicide Prevention Coordinator

Signed: 05/05/2023 09:44

EMERGENCY DEPT NURSE DISCHARGE

Details

Date: May 4, 2023

Location: ANN ARBOR VA MEDICAL CENTER

Written by: BRANDON THOMAS TEMPLETON

Signed by: BRANDON THOMAS TEMPLETON
Date signed: May 4, 2023

Notes

LOCAL TITLE: EMERGENCY DEPT NURSE DISCHARGE
STANDARD TITLE: EMERGENCY DEPT DISCHARGE NOTE
DATE OF NOTE: MAY 04, 2023@19:00 ENTRY DATE: MAY 04, 2023@19:00:36
AUTHOR: TEMPLETON,BRANDON T EXP COSIGNER:
URGENCY: STATUS: COMPLETED

ER Nurse Discharge Template

Time Discharge:May 4,2023

Room Number: 16
Mode of Departure:
Patient left per self Ambulatory
Patient status:
alert, oriented x 3(person, place and time), sensoria clear, independent,
requires no assistance

VITALS:
Temperature:
98.8 F (37.1 C)
PULSE:
89
RESPIRATION:
16
BLOOD PRESSURE:
134/79
PULSE OXIMETRY: 98 done on RA
Current Pain:
0

Pain Scale Used: Numeric Scale

Patient does not have a problem related to pain.

IV-not applicable
Discharge Patient/Family Education: Physician Discharge instructions reviewed
with the patient/significant other and given a printed copy.
Comment: Patient stated he no longer wants to wait for the Risperdal before
discharge.

/es/ BRANDON THOMAS TEMPLETON
REGISTERED NURSE
Signed: 05/04/2023 19:02

EMERGENCY DEPT DISPOSITION

Details

Date: May 4, 2023
Location: ANN ARBOR VA MEDICAL CENTER
Written by: CHRISTINA MARIE MD CUTTER
Signed by: CHRISTINA MARIE MD CUTTER
Date signed: May 10, 2023

Notes

LOCAL TITLE: EMERGENCY DEPT DISPOSITION
STANDARD TITLE: EMERGENCY DEPT DISCHARGE NOTE
DATE OF NOTE: MAY 04, 2023@15:52 ENTRY DATE: MAY 04, 2023@15:52:57
AUTHOR: CUTTER,CHRISTINA MA EXP COSIGNER:
URGENCY: STATUS: COMPLETED

VA Ann Arbor Healthcare System
2215 Fuller Rd.
Ann Arbor MI 48105
FEB 17, 2026

ROCKENHAUS,CONRAD ALAN
35560 GRAND RIVER AVE # 244
FARMINGTN HLS, MICHIGAN 48335

EMERGENCY DEPT. PATIENT DISCHARGE INSTRUCTIONS

You were evaluated in the emergency department. We conducted laboratory studies and you were evaluated by our mental health colleagues. You have been provided with a safety plan that was conducted several days prior to presentation. It will be very important that you continue to follow the safety plan and follow-up at the appointments as scheduled following your recent admission. Please ensure that your mental health team or primary care provider re-assess your Lithium level. Please return to the emergency department if you experience thoughts of hurting yourself/others or any other symptoms that are of concern to you.

For problems call:
The Ann Arbor VA call center 1-734-845-5500 or
1-800-361-8387 ext. 55500

05/05/2023 09:00 ANN SUD-PHD 5
05/12/2023 09:00 ANN SUD-PHD 5

05/16/2023 09:30 ANN SUD-MD 1
05/16/2023 10:00 ANN VVC-VOD SUD-PSY 3
05/19/2023 09:00 ANN SUD-PHD 5
06/13/2023 11:00 ANN VVC-VOD NEURO MD-SHAT
07/06/2023 09:00 CAN WHS ACUPUNCTURE 1
10/12/2023 10:00 CAN PACT 2 YELLOW-MD 2

Influenza Immunization

Resolution:

No influenza vaccine recorded for this season.

Information:

Reminder Term: VA-INFLUENZA IMM SEASON START DATE

Computed Finding: VA-FileMan Date

07/30/2022 value - 3220730

Influenza season start date: 7/30/2022

Influenza vaccine recorded after this date will resolve this reminder.

Refusals

Reminder Term: VA-INFLUENZA IMM SEASONAL VACCINE FORMULATIONS

04/20/2023@16:54 Reason: PATIENT DECISION, expires 05/20/2023.

Patient has refused all vaccines in the group.

04/21/2023@14:07:55 Reason: PATIENT DECISION, expires 05/21/2023.

Comments: Patient states he got in December and it was not documented.

Patient has refused all vaccines in the group.

Immunization: INFLUENZA, UNSPECIFIED FORMULATION

04/20/2023@16:54 Reason: PATIENT DECISION, expires 05/20/2023.

Patient has refused all vaccines in the group.

04/21/2023@14:07:55 Reason: PATIENT DECISION, expires 05/21/2023.

Comments: Patient states he got in December and it was not documented.

Patient has refused all vaccines in the group.

Precautions

Reminder Term: VA-INFLUENZA IMM SEASONAL VACCINE FORMULATIONS

04/20/2023@18:31:09 Reason: UNSTABLE/EVOLVING NEURO DZ, expires 05/20/2023.

Immunization: INFLUENZA, UNSPECIFIED FORMULATION

04/20/2023@18:31:09 Reason: UNSTABLE/EVOLVING NEURO DZ, expires 05/20/2023.

Medication Reconciliation:

The patient received a printed list of their medications (including local
and

remote VA active medications, non-VA medications, medications that expired
or

were discontinued within the last 90 days and pending medication orders).

This list was reviewed-reconciled with the patient. Changes in medications
were discussed with the patient. A printed list of the new or changed
medications was given to the patient. Patient verbalized understanding of
the
changes made.

CHRISTINA MARIE CUTTER MD

Attending Physician, Emergency Medicine

ROCKENHAUS,CONRAD ALAN

SUICIDE PREVENTION SAFETY PLAN REVIEW/DECLINE

Details

Date: May 4, 2023
Location: ANN ARBOR VA MEDICAL CENTER
Written by: ALISA J MELOCHE
Signed by: ALISA J MELOCHE
Date signed: May 4, 2023

Notes

LOCAL TITLE: SUICIDE PREVENTION SAFETY PLAN REVIEW/DECLINE
STANDARD TITLE: SUICIDE PREVENTION NOTE
DATE OF NOTE: MAY 04, 2023@15:47 ENTRY DATE: MAY 04, 2023@15:47:15
AUTHOR: MELOCHE,ALISA J EXP COSIGNER:
URGENCY: STATUS: COMPLETED

Veteran and clinician reviewed prior Safety Plan together; no changes indicated at this time.

Date of current Safety Plan: May 1,2023

/es/ ALISA J MELOCHE LMSW
Psychiatric Social Worker
Signed: 05/04/2023 15:47

EMERGENCY DEPARTMENT MENTAL HEALTH CONSULT

Details

Date: May 4, 2023
Location: ANN ARBOR VA MEDICAL CENTER
Written by: NANCY MARIE BIGELOW
Signed by: NANCY MARIE BIGELOW
Date signed: May 4, 2023

Notes

LOCAL TITLE: EMERGENCY DEPARTMENT MENTAL HEALTH CONSULT
STANDARD TITLE: EMERGENCY DEPT NOTE
DATE OF NOTE: MAY 04, 2023@15:41 ENTRY DATE: MAY 04, 2023@15:41:31
AUTHOR: BIGELOW,NANCY MARIE EXP COSIGNER:
URGENCY: STATUS: COMPLETED

*** EMERGENCY DEPARTMENT MENTAL HEALTH CONSULT Has ADDENDA ***

EMERGENCY DEPT MENTAL HEALTH NOTE

PATIENT IDENTIFYING DATA
Name: ROCKENHAUS,CONRAD ALAN
Age: 41
Gender: MALE
Ethnicity: WHITE
Marital status: DIVORCED

Total time: 90 min including F2F with Pt and care coordination.
Pt was interviewed in the ED together with Nancy Bigelow, NP.

DETAILS OF PRESENTING PROBLEM:

Pt is a 41 y/o male with PPHx including Opioid use d/o on MAT, Bipolar d/o, PTSD, anxiety and depression with multiple past suicide attempts and psychiatric hospitalizations (most recently on AIMH April 26, 2023 to May 1, 2023), who presented to the ED via EMS after an alleged argument with his fiance. Police were called to the residence and Pt reports they gave him a choice of going to jail or coming to the ED.

On interview today he denies any mood disturbance or thoughts of self-harm, suicide, or thoughts of harming others. Pt endorses stable mood since discharge from AIMH on 5-1-2023. He reports more stressors in his relationship since starting couples therapy on 5-2-2023, stating "I feel like she doesn't validate my stressors . . .and she doesn't like when I point this out to the therapist".

The alleged altercation happened today when he says he had a seizure and his fiance got upset and started "yelling at me" and that "I can't be your caretaker" because she could lose her job. The fiance then insisted he lie down after his "seizure", and when he awakened, she called him "pathetic" and "a limp-dick man" and wanted him to leave. The Pt reports he then said "I'm not leaving because I pay the rent".

At that point the fiance called 911, stating that she told the police that he pushed her which Pt denies. Pt then stated that he also called 911 "so they could hear my side of the story". When the police arrived Pt states the police believed that he did not push the fiance but gave him the ultimatum of "going to jail or going to the hospital to appease her". Reports the police told him he could return back to the residence after being evaluated.

MH team informed the Pt that we will need to verify with the police department that he may return home or what the appropriate steps will be from ED. Pt is agreeable and states he lives in Plymouth, MI and the Plymouth Police

Department was called. The Police department verified they have no concerns for him to be discharged and return to his residence.

Denies any issues with sleep or appetite. Reports compliance with medication regimen and intention to attend post discharge appointments; Pt also reports he is planning on attending all f/u appointments including appointment tomorrow.

PSYCHIATRIC HISTORY:

- Per chart, Dx include Bipolar I (fights & over-spending), opioid use d/o related to chronic back pain (now on suboxone), PTSD, depression, anxiety, possible delusional d/o with paranoid and sometimes grandiose delusions.
- Previous treatment in TX, recently established care in SUD-C here.
- Multiple past psych admissions (at least 12), most recently on AIMH from 4-26-2023 to 5-1-2023.
- Multiple past psych med trials including Ambien, Klonopin, Valium, Ativan, Eszopiclone, Zoloft, Remeron, Effexor, Cymbalta, Doxepin, Nortriptyline, Seroquel, Haldol, Risperdal consta, Geodon, Depakote, Lithium, Buspar, Hydroxyzine, Propranolol, Prazosin, Trazodone, Suboxone.
- Per chart, multiple suicide attempts. Pt discussed today's attempt via lying on railroad tracks, fiance found him by tracking his location on his phone then brought him to the ED. Pt reported the most recent attempt prior to today was in Oct 2022 - ingested ~50 Metformin tablets in the context of difficulty adjusting to civilian life after incarceration, argument w/ fianc?e and opiate use.

SUBSTANCE USE HISTORY:

- OUD on MAT with buprenorphine
- denies cannabis use "I quit" since discharging from AIMH unit on 5-1-2023
- denies etoh, denies illicit drug use
- occasional adderall from a friend who has a prescription

PSYCHOSOCIAL HISTORY AND STRESSORS:

Divorced, 3 children (notes from 4-27-2023 indicated Pt has 2 children). Recently moved from TX to MI and lives with his fiance, they've been together x3 years. Current income from 100% VA SC. Per chart, Pt has a BA in computer science, past employment- eb/security/computer consulting business. Currently on supervised release for 3 years after ~40 months of incarceration for computer hacking. Pt cites his fiance and his mother as main social supports. Independent w/ ADLs. Denied access to firearms.

Military Service - Navy 2003-2015

Percent SC: 100

Rated disabilities:

SCARS 0% SC

BIPOLAR DISORDER 70% SC

TRAUMATIC BRAIN DISEASE 10% SC

LIMITED FLEXION OF THIGH 0% SC

LIMITED MOTION OF ARM 0% SC

HYPERTENSIVE VASCULAR DISEASE 0% SC

SCARS 0% SC

SCARS 0% SC

LIMITED MOTION OF ARM 0% SC

SLEEP APNEA SYNDROMES 50% SC

MIGRAINE HEADACHES 30% SC
LIMITED FLEXION OF THIGH 10% SC
SCARS 0% SC
LUMBOSACRAL OR CERVICAL STRAIN 0% SC
PARALYSIS OF SCIATIC NERVE 10% SC
LUMBOSACRAL OR CERVICAL STRAIN 10% SC
IRRITABLE COLON 0% SC
LIMITED FLEXION OF KNEE 10% SC
TINNITUS 10% SC
SEIZURE DISORDER 100% SC

MEDICAL HISTORY:

Per chart:

- Seizure disorder
- Sleep apnea, not using CPAP ("ex-wife stole it")
- Migraine
- Sciatica
- Chronic back pain
- Tendinitis
- Hypertension
- Neck pain
- Atopic dermatitis
- Hearing loss
- Morbid obesity
- Diabetes
- Allergic rhinitis
- Pulmonary vein injury
- GERD

Active Outpatient Medications (including Supplies):

-
- 1) ACETAMINOPHEN 250/ASA 250/CAFF 65MG TAB TAKE 2 ACTIVE
TABLETS BY MOUTH EVERY DAY AS NEEDED FOR HEADACHE
 - 2) ACETAMINOPHEN 500MG TAB TAKE 1 TABLET BY MOUTH TWICE ACTIVE
A DAY AS NEEDED FOR PAIN
 - 3) BUPRENORPHINE 8MG/NALOXONE 2MG SL TAB DISSOLVE 1 ACTIVE
TABLET UNDER THE TONGUE EVERY MORNING - THIS RX FOR
5-1-23 TO 5-16-23
 - 4) FAMOTIDINE 20MG TAB TAKE ONE TABLET BY MOUTH TWICE A ACTIVE
DAY FOR HEARTBURN
 - 5) FOLIC ACID 1MG TAB TAKE ONE TABLET BY MOUTH ONCE ACTIVE
DAILY
 - 6) LITHIUM CARBONATE 300MG CAP TAKE FOUR CAPSULES BY ACTIVE
MOUTH AT BEDTIME FOR MENTAL HEALTH
 - 7) PILL SPLITTER-EA USE SPLITTER -- AS DIRECTED TO SPLIT ACTIVE
TABLETS
 - 8) PRAZOSIN HCL 1MG CAP TAKE THREE CAPSULES BY MOUTH ACTIVE
TWICE A DAY FOR NIGHTMARES OR SLEEP DISRUPTION
 - 9) RISPERIDONE 1MG TAB TAKE ONE-HALF TABLET BY MOUTH ACTIVE
THREE TIMES A DAY AS NEEDED FOR ANXIETY AND
DIFFICULTY SLEEPING
 - 10) TRAZODONE HCL 100MG TAB TAKE ONE TABLET BY MOUTH AT ACTIVE
BEDTIME FOR SLEEP

11) VITAMIN B COMPLEX/VITAMIN C CAP/TAB TAKE 1 TABLET BY ACTIVE
MOUTH ONCE DAILY FOR VITAMIN SUPPLEMENT

MENTAL STATUS EXAMINATION:

Alert: sitting up in bed in VA attire good hygiene, good eye contact, pleasant and cooperative in interview

Orientation: A & O x 4

Psychomotor: no psychomotor agitation/retardation. No tremors or tics noted.

Speech: normal rate, rhythm, loud volume x times

Mood: "fine"

Affect: broad, reactive; mood congruent

Thought processes: overall logical, linear, goal-directed

Concentration: appropriate to interview; no deficits noted

Thought content: Denies SI/HI intent or plan. Denies SIB thoughts/urges.

Hallucinations: Denies AVH. No evidence of responding to internal stimuli or thought blocking.

Delusions: No paranoid or delusional statements elicited or expressed.

Insight: limited-to-fair

Judgement: limited-to-fair

SUICIDE RISK:

See CSRE

HOMICIDE/VIOLENCE RISK: No. Plymouth Police Department contacted by MH team to verify they have no safety concerns to discharge Pt to return to the residence; Plymouth PD have no concerns for Pt to be discharged.

DSM V DIAGNOSES:

-mood disorder unspecified d/o

-bipolar d/o by history

-OUD severe on MAT with buprenorphine

-chronic PTSD by history

-Delusional d/o by history

-cannabis use daily (recently quit)

IMPRESSION:

Pt is a 41 y/o male with psychiatric history of bipolar d/o, chronic PTSD, delusional d/o, OUD severe on MAT, past suicide attempts (most recent report of S.A is lying on railroad tracks on 4-26-2023), numerous inpatient psychiatric admissions (most recent AIMH 4-26-2023 to 5-1-2023), has received MH treatment at the inpatient and outpatient levels of care who presents to AAVAED via EMS for purported SI after argument with fiancée.

Pt denies low mood, SI, or HI now or at any time since discharge from AIMH unit on 5-1-2023 and reports that the police gave him the choice of going to jail or going to get evaluated in the ED after 911 was called after argument with fiancée. Pt chose to come to ED to be evaluated and "appease" his fiancée. The Plymouth Police Dept. was contacted by the ED MH team and report each party said they pushed the other and they also confirm there are no safety concerns for the Pt to return to residence.

Pt provided his version of events that led to ED presentation and in summary:

he and fiancée had argument today after he had a "seizure" and she said she could not be his caretaker. The argument escalated to the fiancée verbally insulting him and telling him to leave; the Pt said he will not leave b/c he pays the rent. Each party called 911 and the police essentially sent the Pt to the ED to separate them for a cooling off period.

Pt is pleasant t/o the interview with all ED staff. Pt presentation appears to chronically dramatic/grandiose which may be consistent with mania, emotional distress, personality d/o, mood d/o, and/or delusional disorder and may be obfuscated by OUD pathology. He was recently observed on AIMH unit x 4 days, and while he was primarily admitted for a depressive episode, suspicion was maintained for a personality contribution. This was reinforced by his rapid improvement without specific intervention. He was maintained on his home lithium, prazosin, and trazodone. Additionally low-dose risperidone was added to target depression, anxiety, and insomnia. Suboxone dosing was not altered during the course of his stay.

Pt does not meet criteria for involuntary admission. Due to lack of acute psychiatric symptoms, available support, independence with ADLs, willingness to seek help, future-orientation, engagement with outpatient psychiatry for medication management and no safety concerns will discharge Pt to home with outpatient f/u already in place s/p discharge from AIMH on 5-1-2023

PLAN:

- Patient is safe for d/c to home with outpatient follow up.
- F/u at his next scheduled outpatient appointment 5-5-2023
- Reviewed Safety Plan and provided with copy.
- Counseling provided re: alternative coping strategies, attending f/u MH appointments and taking medication as prescribed for best outcomes for MH recovery and maintenance.
- Provided Veteran's Crisis Line #. Pt educated re worsening s/sx and advised to call if these occur.
- Pt verbalized understanding and agreement with plan.

This case discussed with ED attending Dr. Cutter and Alisa Meloche LMSW

/es/ NANCY MARIE BIGELOW
Nurse Practitioner, Psychiatry
Signed: 05/04/2023 16:19

Receipt Acknowledged By:

05/05/2023 07:59 /es/ JESICA LEIGH KALMBACH
Staff Psychologist
05/04/2023 16:25 /es/ ALISA J MELOCHE LMSW
Psychiatric Social Worker
05/08/2023 09:38 /es/ MARC A. OLSON, MD
Staff Physician, Substance Abuse Clinic

05/04/2023 ADDENDUM

STATUS: COMPLETED

Writer called back by Emergency Department Physician regarding the care of Mr. Rockenhaus. Previously seen by ER Mental Health Consult team (see notes above), and cleared for d/c. Patient Mr. Rockenhaus reportedly called fiancée (Adrienne Blair 734-585-6277) to tell her he was coming home. Ms. Blair called into the

AAVA ED to report that she had safety concerns (text messages saying that he was going to plan on harming himself if SHE had to leave house to go to the hospital for her own mental health care). Also reported suicide attempt of laying on tracks (of note, this was prior to last admission, and he has not had any suicidal attempts since most recent d/c from inpt admission on 5/1). Ms. Blair was hyperverbal on the phone. Upon re-evaluation, pt continues to deny any SI/HI. He was offered voluntary admission to AIMH for further evaluation and management. Pt declined. Pt does not meet criteria for involuntary admission as he continues to deny symptoms.

Safety plan reviewed with patient. He has close follow-up with SUD-C Psychologist visit tomorrow 5/5 at 9am. Pt reported if he did have symptoms of SI/HI overnight he would discuss at visit tomorrow.

Originally safety planned with patient for him to go to a hotel for the night, so he would be able to make it to 9am appt at AAVA tomorrow morning, and so that he could allow some time for domestic argument to resolve. When writer left to print out safety plan, writer returned to patient talking to Ms. Blair on the phone, saying they had made up and she will pick him up. Writer again reinforced benefits of spending the night apart and recommended hotel for the evening. Writer offered patient 1mg Risperidone to help relax prior to d/c from ED.

/es/ MICHAEL JAMES FRANKLIN
Resident, Psychiatry
Signed: 05/04/2023 18:32

/es/ ANUSHA RANGANATHAN MD
Attending Physician, Psychiatry
Cosigned: 05/05/2023 10:02

SUICIDE RISK EVALUATION - COMPREHENSIVE

Details

Date: May 4, 2023
Location: ANN ARBOR VA MEDICAL CENTER
Written by: ALISA J MELOCHE
Signed by: ALISA J MELOCHE
Date signed: May 4, 2023

Notes

LOCAL TITLE: SUICIDE RISK EVALUATION - COMPREHENSIVE
STANDARD TITLE: SUICIDE PREVENTION RISK ASSESSMENT SCREENING NOT
DATE OF NOTE: MAY 04, 2023@15:32 ENTRY DATE: MAY 04, 2023@15:32:12
AUTHOR: MELOCHE,ALISA J EXP COSIGNER:
URGENCY: STATUS: COMPLETED

*** SUICIDE RISK EVALUATION - COMPREHENSIVE Has ADDENDA ***

Comprehensive Suicide Risk Evaluation

This is an update to an existing suicide risk evaluation.
The validity of the information contained within this evaluation is not in question.

Suicidal Ideation

The most recent thoughts of engaging in suicide-related behavior were within the past 30 days.

Pt denies SI since he was hospitalized on AIMH 4/26/23.

The Veteran had suicidal intent at the time of the most recent ideation.

Description of intent: acting on SI on 4/26/23 by lying on railroad tracks. Denies SI, plan or intent since then.

The Veteran had a suicide plan at the time of the most recent ideation.

Describe: acting on SI on 4/26/23 by lying on railroad tracks. Denies SI, plan or intent since then.

The most recent suicidal ideation was the most severe ideation within the last 30 days.

The Veteran does not have access to lethal means (firearms)

The Veteran does not have access to other lethal means.

Suicidal Behavior

The Veteran has not made any suicide attempts since the last VA Comprehensive Suicide Risk Evaluation was completed.

The Veteran did not report any prior preparatory behaviors that have not been previously documented.

Warning Signs

The following warning signs are currently present for the Veteran:

None noted

Risk Factors

History of suicidal behavior(s)

Comment: multiple, most recent attempt 4/26/23

Recent psychosocial stressors

Please Describe: relationship problems

History of mental health hospitalization

Please Describe: multiple, most recently April 2023 on AIMH

Psychological conditions or symptoms

Please Describe: bipolar d/o, ptsd, anxiety, depression, opioid use d/o

Protective Factors and Reasons for Living

Access to and engagement with health care

Access to and engagement with mental health care

Reports motivation for mental health treatment

Has meaningful family relationships

Comment: mother

Hope for the future

Clinical Impressions:

The clinical impression of acute risk is Low ACUTE Risk.

As evidenced by: Denies SI, plan or intent.

The clinical impression of chronic risk is High CHRONIC Risk.

As evidenced by: Multiple past suicide attempts and psych admissions, long MH/SUD history, limited coping skills, impulsivity, limited support system.

Suicide Risk Mitigation Plan:

This treatment and care plan was developed in collaboration with the Veteran.

This treatment and care plan was developed in collaboration with additional healthcare providers.

List: Nancy Bigelow, NP

Risk Mitigation Plan:

Strategies for Managing Risk if the Veteran is Currently in the EMERGENCY DEPARTMENT/URGENT CARE CENTER:

Suicide Prevention Coordinator was not alerted for consideration of a Patient Record Flag Category I High Risk for Suicide.

Complete or update Veteran's safety plan

Inform Veteran that they will receive follow-up phone contact until engaged in mental health care.

Obtain additional information from collateral sources

Comment: Plymouth PD

Provide Veteran with phone number for Veteran's Crisis Line: Dial 988 (Press 1), Text to 838255, or Chat

Re-evaluation:

Due to the dynamic nature of some warning signs, risk and protective factors, suicide risk should be routinely re-evaluated. These risk management strategies were chosen to address Veteran's current presentation and feasible treatment options within the system of care. This plan should be re-evaluated over time.

/es/ ALISA J MELOCHE LMSW

Psychiatric Social Worker

Signed: 05/04/2023 15:46

05/05/2023 ADDENDUM

STATUS: COMPLETED

Thank you for completing this CSRE and alerting and collaborating with Suicide Prevention team. The Veteran currently has HRS PRF in chart.

Treatment providers should continue to assess for suicide risk during clinical contacts. Suicide Prevention team will continue to monitor Veteran's care, per HR PRF protocol. SP team appreciates the ongoing collaboration & coordination of care by Veteran's treatment team.

/es/ MARY PAT TUCKER, LISW-S

Suicide Prevention Coordinator

Signed: 05/05/2023 09:44

EMERGENCY DEPT FLOWSHEET

Details

EMERGENCY DEPARTMENT TRIAGE

Details

Date: May 4, 2023
Location: ANN ARBOR VA MEDICAL CENTER
Written by: BRANDON THOMAS TEMPLETON
Signed by: BRANDON THOMAS TEMPLETON
Date signed: May 4, 2023

Notes

LOCAL TITLE: EMERGENCY DEPARTMENT TRIAGE
STANDARD TITLE: NURSING EMERGENCY DEPT NOTE
DATE OF NOTE: MAY 04, 2023@14:34 ENTRY DATE: MAY 04, 2023@14:34:50
AUTHOR: TEMPLETON, BRANDON T EXP COSIGNER:
URGENCY: STATUS: COMPLETED

*** EMERGENCY DEPARTMENT TRIAGE Has ADDENDA ***

EMERGENCY DEPT NURSING TRIAGE

Check-in Date/Time: 05/04/23 13:55

Triage Date/Time: MAY 04, 2023 14:34

The patient was asked if in the last 14 days they have had new onset of any COVID-19 symptoms. They report the following:

No symptoms

Within the past 14 days, the patient reports no exposure to someone with a febrile/respiratory illness or someone with a known or suspected case of COVID-19 (within 6 feet for > 15 minutes).

Result:

Screen is negative.

Patient age:41 Sex: MALE

On arrival patient was: AMBULANCE

Patient phone number: PATIENT PHONE

Allergies: SULFA DRUGS, DARUNAVIR

Subjective/Chief Complaint:

Pt to ED w/ cc of SI from home after altercation with wife and local PD were called.

Objective:

Current Medications:

Active Outpatient Medications (including Supplies):

Active Outpatient Medications

Status

-
-
- 1) ACETAMINOPHEN 250/ASA 250/CAFF 65MG TAB TAKE 2 ACTIVE
TABLETS BY MOUTH EVERY DAY AS NEEDED FOR HEADACHE
 - 2) ACETAMINOPHEN 500MG TAB TAKE 1 TABLET BY MOUTH TWICE ACTIVE
A DAY AS NEEDED FOR PAIN
 - 3) BUPRENORPHINE 8MG/NALOXONE 2MG SL TAB DISSOLVE 1 ACTIVE
TABLET UNDER THE TONGUE EVERY MORNING - THIS RX FOR
5-1-23 TO 5-16-23
 - 4) FAMOTIDINE 20MG TAB TAKE ONE TABLET BY MOUTH TWICE A ACTIVE
DAY FOR HEARTBURN
 - 5) FOLIC ACID 1MG TAB TAKE ONE TABLET BY MOUTH ONCE ACTIVE
DAILY
 - 6) LITHIUM CARBONATE 300MG CAP TAKE FOUR CAPSULES BY ACTIVE
MOUTH AT BEDTIME FOR MENTAL HEALTH
 - 7) PILL SPLITTER-EA USE SPLITTER -- AS DIRECTED TO SPLIT ACTIVE
TABLETS
 - 8) PRAZOSIN HCL 1MG CAP TAKE THREE CAPSULES BY MOUTH ACTIVE
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DIFFICULTY SLEEPING
 - 10) TRAZODONE HCL 100MG TAB TAKE ONE TABLET BY MOUTH AT ACTIVE
BEDTIME FOR SLEEP
 - 11) VITAMIN B COMPLEX/VITAMIN C CAP/TAB TAKE 1 TABLET BY ACTIVE
MOUTH ONCE DAILY FOR VITAMIN SUPPLEMENT

Current Problems: ACTIVE PROBLEMS

Vital Signs *

Temperature

98.8 F (37.1 C)

Pulse

100

Respirations

16

Blood Pressure

138/82

Pain scale recorded:

0

Pulse Oximetry 98 Room Air

SEPSIS SCREENING

Is the patient's history suggestive of a new infection?

No

Sepsis screening not indicated, will continue to monitor.

The patient is not a fall risk.

Suicide Screen:

Columbia Suicide Severity Rating Scale (C-SSRS) screener

Specific method of suicide recently considered; this POSITIVE answer requires same-day completion of a Suicide Risk Evaluation-Comprehensive

1. Over the past month, have you wished you were dead or wished you could go to sleep and not wake up?

Yes

2. Over the past month, have you had any actual thoughts of killing yourself?

Yes

3. Over the past month, have you been thinking about how you might do this?

Yes

4. Over the past month, have you had these thoughts and had some intention of acting on them?

No

5. Over the past month, have you started to work out or worked out the details of how to kill yourself?

No

6. If yes, at any time in the past month did you intend to carry out this plan?

Response not required due to responses to other questions.

7. In your lifetime, have you ever done anything, started to do anything, or prepared to do anything to end your life (for example, collected pills, obtained a gun, gave away valuables, went to the roof but didn't jump)?

Yes

8. If YES, was this within the past 3 months?

No

Patient had a C-SSRS that was positive. Warm handoff for CSRE.

Hand off to: Russ, ED tech

HOMICIDE SCREENING

Have you had any recent thoughts of hurting someone else?

No

SUSPECTED ASSAULT, ABUSE OR NEGLECT SCREEN

The patient has not exhibited any clinical signs and/or stated that

he/she has not been assaulted, abuse or neglected.

Influenza vaccine not due at this time.

Emergency Severity Index (ESI) level
Level 2

PATIENT PLACEMENT:
placed patient in room
ROOM NUMBER: 2

/es/ BRANDON THOMAS TEMPLETON
REGISTERED NURSE
Signed: 05/04/2023 14:37

05/04/2023 ADDENDUM STATUS: COMPLETED
Patient brought back to ED room and placed on 1:1 upon arrival to ED at 13:55
while writer was at CT with another patient. EMS gave hand off to Russ, ED tech
upon arrival and continues to be on 1:1 at this time.

/es/ BRANDON THOMAS TEMPLETON
REGISTERED NURSE
Signed: 05/04/2023 14:54

EMERGENCY DEPT

Details

Date: May 4, 2023
Location: ANN ARBOR VA MEDICAL CENTER
Written by: CHRISTINA MARIE MD CUTTER
Signed by: CHRISTINA MARIE MD CUTTER
Date signed: May 4, 2023

Notes

LOCAL TITLE: EMERGENCY DEPT
STANDARD TITLE: EMERGENCY DEPT NOTE
DATE OF NOTE: MAY 04, 2023@14:29 ENTRY DATE: MAY 04, 2023@14:29:14
AUTHOR: CUTTER,CHRISTINA MA EXP COSIGNER:
URGENCY: STATUS: COMPLETED

*** EMERGENCY DEPT Has ADDENDA ***

HISTORY OF PRESENT ILLNESS

Patient is a 41-year-old male with a past medical history pertinent for
bipolar disorder, depression, suicidal ideation/attempt, anxiety, PTSD, opioid

use disorder (on Suboxone) with recent inpatient mental health admission who presents for evaluation. Patient reports that he has been doing well without suicidal or homicidal ideation. He cites a verbal altercation with his significant other earlier today. He consequently presents for evaluation by EMS. No petition was completed. He denies any current suicidal or homicidal ideation. He denies any recent thoughts or suicide attempt. He denies any current alcohol or substance use. He has been taking all medications as prescribed without additional doses of prescribed or over-the-counter medications. He denies headache, changes in vision, new numbness and weakness in his arms or legs. He denies fever, chills, intractable nausea/vomiting/diarrhea, dysuria, cough, sore throat. He denies chest pain, shortness of breath. No abdominal pain. He denies recent seizure; no urinary/stool incontinence, no tongue biting. He has been taking all medications as prescribed. He denies recent incurred trauma including head trauma. No signs suggestive of blood loss.

REVIEW OF SYSTEMS

Negative or non-contributory except as noted in the above HPI.

PAST MEDICAL HISTORY, PAST SURGICAL HISTORY, SOCIAL HISTORY, FAMILY HISTORY
Reviewed (as pertinent)

MEDICATIONS

Active Outpatient Medications (including Supplies):

ACETAMINOPHEN 250/ASA 250/CAFF 65MG TAB TAKE 2 TABLETS BY MOUTH EVERY DAY AS NEEDED FOR HEADACHE ACTIVE
ACETAMINOPHEN 500MG TAB TAKE 1 TABLET BY MOUTH TWICE A DAY AS NEEDED FOR PAIN ACTIVE
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FAMOTIDINE 20MG TAB TAKE ONE TABLET BY MOUTH TWICE A DAY FOR HEARTBURN ACTIVE
FOLIC ACID 1MG TAB TAKE ONE TABLET BY MOUTH ONCE DAILY ACTIVE
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PILL SPLITTER-EA USE SPLITTER -- AS DIRECTED TO SPLIT TABLETS ACTIVE
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TRAZODONE HCL 100MG TAB TAKE ONE TABLET BY MOUTH AT BEDTIME FOR SLEEP ACTIVE
VITAMIN B COMPLEX/VITAMIN C CAP/TAB TAKE 1 TABLET BY MOUTH ONCE DAILY FOR VITAMIN SUPPLEMENT ACTIVE

ALLERGIES

SULFA DRUGS, DARUNAVIR

PHYSICAL EXAMINATION

Vital signs and nursing notes reviewed.

General: patient is alert, cooperative, and in no acute distress speaking in full sentences with ease
Skin: warm, dry; no visible lacerations
HEENT: (Head) atraumatic; (Eyes) extraocular movements intact; (Ears) no drainage visualized; (Sinuses/Nose) no epistaxis; (Neck/Trachae) neck is supple, trachae is midline; (Throat) oropharynx with slight dry mucous membranes; no visible tongue biting
Pulmonary: decent aeration throughout; normal work of breathing
Cardiovascular: regular rate (improved from triage vitals); extremities well perfused
Abdomen: soft, non-distended, non-tender; no rebound or guarding
Extremities: moving all extremities
Psychological: denies suicidal or homicidal ideation
Neurologic: (General) alert and oriented, good concentration, can follow commands across midline. Speech is fluent without dysarthria or word-finding difficulty. No nuchal rigidity. (Cranial nerves) Extraocular movements are intact. Face is symmetric at rest and activation. Facial sensation is intact V1 through V3. Palate and tongue are midline. Sternocleidomastoid and trapezius have full power. (Motor) moves all extremities. Muscle strength intact to hand grasp, biceps/triceps, knee flexion/extension. (Cerebellar) finger to nose intact. (Sensation) light touch sensation intact in the bilateral upper and lower extremities

MEDICAL DECISION MAKING

Patient is a 41-year-old male with a past medical history significant for bipolar disorder, depression, suicidal ideation/attempt, anxiety, PTSD, opioid use disorder (on Suboxone) with recent inpatient mental health admission who presents for evaluation. Upon initial evaluation, patient was in no acute distress. His initial vital sign assessment is notable for being afebrile with a borderline fast heart rate. His examination was as above. Initial management strategy included initiation of suicide precautions, laboratory assessment, mental health consultation. Laboratory assessment was notable for absence of leukocytosis, normal hemoglobin, normal platelet count, normal renal function, normal hepatic function panel, toxicologic evaluation was notable for a negative acetaminophen, negative salicylates, negative ethanol, urine toxicologic evaluation was evidence of cannabinoids, urinalysis without signs suggestive of infection. TSH normal. EKG without evidence acute ST-T changes. Mental health promptly presented for evaluation. Their evaluation was without evidence of safety concerns. This was corroborated by conversation with police. Patient continues to deny any safety concerns upon serial evaluation and upon conversation with disparate care team members. Consequently, patient was determined appropriate for discharge home by mental health with outpatient management. Patient was provided with a copy of his safety plan. He has multiple outpatient appointments that are currently scheduled following his inpatient admission. Patient was instructed to follow the safety plan, outpatient management plan as scheduled. Patient continued to deny safety concerns. Upon plan for discharge, nursing notified care team that patient significant other called and was concerned about patient safety. At this time, patient care was signed out to Dr. Byrne pending conversation with patient significant other and safety re-evaluation. Please see his note for the consequent medical decision-making and ultimate disposition determination for this patient.

DISPOSITION

Pending

/es/ CHRISTINA MARIE CUTTER MD
Attending Physician, Emergency Medicine
Signed: 05/04/2023 16:51

05/04/2023 ADDENDUM STATUS: COMPLETED
Ongoing physician addendum:

In brief, this is a 41-year-old who was brought here for potential suicidal ideation or behavioral disturbance after domestic dispute with his girlfriend/fiance. He has been evaluated by my predecessor as well as the mental health team.

He had been discharged, when his fiance heard that that was the case. She called here frantic. She was yelling, with pressured speech, about his previous suicidal ideation and statements. She is very vague about the time course. She says that she does not think that he is safe to come home.

Given this, I called the psychiatry service again in the light of new collateral information and we will commune as to the appropriate next steps.

Psych saw him once again. They got more collateral and did more safety planning. He will be discharged. He will get a milligram of oral Risperdal prior to that.

/es/ BRENDAN TERENCE BYRNE MD
Attending Physician, Emergency Department
Signed: 05/04/2023 18:25

SUICIDE RISK MANAGEMENT FOLLOW-UP

Details

Date: May 4, 2023
Location: ANN ARBOR VA MEDICAL CENTER
Written by: ADRIANA A LIBURDI
Signed by: ADRIANA A LIBURDI
Date signed: May 4, 2023

Notes

LOCAL TITLE: SUICIDE RISK MANAGEMENT FOLLOW-UP
STANDARD TITLE: SUICIDE PREVENTION NOTE
DATE OF NOTE: MAY 04, 2023@11:49 ENTRY DATE: MAY 04, 2023@11:50:02
AUTHOR: LIBURDI,ADRIANA A EXP COSIGNER:
URGENCY: STATUS: COMPLETED

Suicide Risk Management Follow-Up
PURPOSE: Suicide Prevention High Risk Follow-Up
Contacted Veteran for follow-up related to:
Inpatient psychiatric unit discharge

High Risk for Suicide Patient Record Flag (HRS-PRF)

OUTREACH OUTCOME

Attempt to contact made at the following times:

11:50 a.m. 5/4/23

Left a HIPPA compliant voice mail message reminded veteran of appoingment on 5/5/23 at 9 a.m. with phone number and request to call back.

WELLNESS CHECK DETERMINATION

Based on brief chart review or recent contact with the Veteran, there is no indication of imminent risk for suicide or danger to others or a medical emergency at this time. A wellness check is not clinically indicated at this time.

/es/ Adriana A Liburdi, LMSW
Suicide Prevention Case Manager
Signed: 05/04/2023 11:51

DISCHARGE FOLLOW UP CALL

Details

Date: May 3, 2023
Location: DORIS MILLER VAMC
Written by: MARY J FILER
Signed by: MARY J FILER
Date signed: May 3, 2023

Notes

LOCAL TITLE: DISCHARGE FOLLOW UP CALL
STANDARD TITLE: VIST TELEPHONE ENCOUNTER NOTE
DATE OF NOTE: MAY 03, 2023@11:32 ENTRY DATE: MAY 03, 2023@11:32:55
AUTHOR: FILER,MARY J EXP COSIGNER:
URGENCY: STATUS: COMPLETED

DISCHARGE FOLLOW UP CALL

Caller was unable to reach the patient via phone.
Left message for patient.

/es/ Mary Jo Filer RN BSN
Registered Nurse
Signed: 05/03/2023 11:33

LETHAL MEANS SAFETY KIT LETTER

Details

Date: May 2, 2023
Location: ANN ARBOR VA MEDICAL CENTER

Written by: TREAVA COOK
Signed by: TREAVA COOK
Date signed: May 2, 2023

Notes

LOCAL TITLE: LETHAL MEANS SAFETY KIT LETTER
STANDARD TITLE: SOCIAL WORK NOTE
DATE OF NOTE: MAY 02, 2023@14:11 ENTRY DATE: MAY 02, 2023@14:11:38
AUTHOR: COOK,TREAVA EXP COSIGNER:
URGENCY: STATUS: COMPLETED

Lethal Means Safety Kit (Mailed)

A home safety kit (Lethal Means Safety Kit) was provided mailed to the Veteran to promote use of suicide risk mitigation tools.

1. The kit included the following items
1. Letter providing information about the Home Safety Kit
2. Crisis Line Wallet Card
3. Medication Disposal Envelope
4. Gun Lock
5. Reducing Firearm and Other Household Safety Risks Brochure
6. Know the Warning Signs Postcard
7. Building Relationship Health and Safety for Life Flyer

DISCHARGE FOLLOW UP CALL

Details

Date: May 2, 2023
Location: DORIS MILLER VAMC
Written by: MARY J FILER
Signed by: MARY J FILER
Date signed: May 2, 2023

Notes

LOCAL TITLE: DISCHARGE FOLLOW UP CALL
STANDARD TITLE: VIST TELEPHONE ENCOUNTER NOTE
DATE OF NOTE: MAY 02, 2023@09:48 ENTRY DATE: MAY 02, 2023@09:48:13
AUTHOR: FILER,MARY J EXP COSIGNER:

URGENCY:

STATUS: COMPLETED

DISCHARGE FOLLOW UP CALL

Caller was unable to reach the patient via phone.

Left message for patient.

/es/ Mary Jo Filer RN BSN

Registered Nurse

Signed: 05/02/2023 09:48

Discharge Summary

Details

Location: ANN ARBOR VA MEDICAL CENTER

Date admitted: None recorded

Date discharged: None recorded

Discharged by: MICHAEL JAMES FRANKLIN

Summary

LOCAL TITLE: Discharge Summary

STANDARD TITLE: DISCHARGE SUMMARY

DICT DATE: MAY 01, 2023@14:19 ENTRY DATE: MAY 01, 2023@14:19:15

DICTATED BY: FRANKLIN,MICHAEL JA ATTENDING: RANGANATHAN,ANUSHA MD

URGENCY: routine

STATUS: COMPLETED

AIMH DISCHARGE SUMMARY

PRIMARY DIAGNOSIS:

Suicidal ideation, plan, and preparatory behavior

Adjustment Disorder vs. bereavement.

SECONDARY DIAGNOSES:

Bipolar 1 disorder

PTSD

Opioid dependence

Personality disorder, unspecified

Daily cannabis use

PROCEDURES:

None

ID:

41-year-old male with past psychiatric history of Bipolar 1 disorder with depressive episodes and past suicide attempts, unspecified anxiety disorder, PTSD, and opioid use disorder on buprenorphine maintenance therapy who was admitted for worsening depression and recent suicide attempt.

HISTORY OF PRESENT ILLNESS:

The patient is a 41-year-old male with a past psychiatric history significant

for with depressive episodes and past suicide attempts, unspecified anxiety disorder, PTSD, and opioid use disorder on buprenorphine maintenance therapy who presents to the Ann Arbor VA ED on 4/26/2023 after an attempted suicide.

The patient says he was pushed "over the edge" last night when he "could not finish" during sexual intercourse with his fiancée due to his chronic lower back pain. An argument with his fiancée ensued. After this, he proceeded to the train tracks near his home and laid there for 10 minutes in anticipation of a train striking and killing him. He believed the next train would arrive in 10 to 15 minutes. He stated he did not inform anyone else of his plan because he "did not want anyone to stop [him]." However, he admits he "stupidly" neglected to turn off his phone location, which allowed his fiancée to track him and put an end to the suicide attempt. In talking to his fiancée, he was tearful and acutely distressed, though easily persuaded off of the rail track. She then took him to our ED.

The patient says he has been feeling down over the past couple weeks due to multiple stressors. He reports unremitting lower back pain and difficulty receiving adequate treatment because "no one believes me." He also reports having lost several friends to suicide; the most recent friend died three weeks ago. He expressed guilt for not "seeing the signs" and laments what he "could have done" for them. Additionally, his mother-in-law passed away several weeks ago, as well. He says his fiancée, Adrienne, is the "only reason" he is happy and endorses feeling that he has let her down because he "can't keep it together."

The patient moved to Michigan (where his fiancée from) two months ago after getting approval from his federal probation officer. He was released from prison in October of 2022 (incarcerated for computer hacking and dark web involvement, stating he was a "notorious" computer hacker, targeted by the federal government) and reports having no "real friends" here.

Additional psychiatric symptoms include stated anxiety, described as an internal sensation of restlessness. Denies excessive worry. He believes his last episode of mania, described as having excessive energy, sleeping little, spending excessively, and engaging in high risk activities, occurred while he was in the service. He denies recurrent symptoms presently. He reports hearing muffled male and female voices when no one is around that may represent auditory hallucinations.

Other than smoking his marijuana pen on a daily basis (uses 1 cartridge of 60% THC daily) to help control his anxiety and seizures, he denies drug use. He also denies alcohol and tobacco use. He states that he takes clonazepam 0.5 mg most nights to help him sleep, and Adderall obtained from an acquaintance "once in a while" because he believes he has ADHD.

PAST PSYCHIATRIC HISTORY:

Diagnoses

- Bipolar 1 disorder
- Unspecified anxiety
- PTSD
- Opioid use disorder
- Daily cannabis use

- Possible delusional disorder

Hospitalizations

- Multiple for mania, depression, and suicidal ideation
- Last in October 2022 after suicide attempt

Past and current psychiatric treatment:

Presently receiving psychiatric care with Dr. Thomas Park through the SUD clinic, established in 4/2023. Lithium, 1200 mg daily; buprenorphine 8 mg/naloxone 2 mg, daily; prazosin 3 mg twice daily; trazodone 100 mg nightly

Medication trials:

- Sertraline
- Duloxetine, venlafaxine
- Mirtazapine
- Doxepin, nortriptyline
- Trazodone
- Haloperidol, quetiapine, ziprasidone
- Risperdal Consta
- Depakote, lithium
- Buspirone, hydroxyzine, propranolol
- Clonazepam, diazepam, lorazepam
- Prazosin
- Zolpidem, eszopiclone
- Suboxone

PAST MEDICAL HISTORY:

- Hypertension
- Obesity
- Type 2 diabetes
- GERD
- Sleep apnea
- Pulmonary vein injury
- Traumatic brain injury/posttraumatic seizures
 - Recent work-up unrevealing, the patient reports chronic history of levetiracetam use since his TBI
- Migraine headaches
- Sciatica
- Chronic neck and back pain
- Tendinitis
- Allergic rhinitis
- Atopic dermatitis
- Hearing loss

PCP: Rubina Shaikh

ALLERGIES:

FACILITY	ALLERGY/ADR
-----	-----

11324^CLNCL/HLTH DAT REPT EFF 030109^200CHNONSTEROIDAL ANTI-INFLAMMATORY
11324^CLNCL/HLTH DAT REPT EFF 030109^200CHOPIOID ANALGESICS

11324^CLNCL/HLTH DAT REPT EFF 030109^200CHSULFA DRUGS
549^DALLAS VA MEDICAL CENTER^549 SULFA DRUGS
674^OLIN E. TEAGUE VET CENTER^674 DARUNAVIR
674^OLIN E. TEAGUE VET CENTER^674 NONSTEROIDAL ANTI-INFLAMMATORY
674^OLIN E. TEAGUE VET CENTER^674 SULFA DRUGS
SULFA DRUGS, DARUNAVIR

SIGNIFICANT FINDINGS ON ADMISSION:

MENTAL STATUS EXAM:

APPEARANCE: Appears stated age, sitting on med, wearing hospital clothing

BEHAVIOR: Cooperative to interview, makes appropriate, though occasionally intense, eye contact; no tics or psychomotor slowing

SPEECH: Rate at time slows; regular prosody and articulation; loud volume.

MOOD: "Afraid"

AFFECT: Labile range of emotions, inappropriate reactivity

THOUGHT PROCESS: Circumstantial

THOUGHT CONTENT: Psychosocial stressors, depressive symptoms, possible persecutory and grandiose delusions, report suicidal ideation with recent plan and intent to lie on rail track, denies homicidal ideation

PERCEPTUAL DISTURBANCES: Reports recent auditory hallucinations denies visual hallucinations

COGNITION: Intact to interview, not formally assessed

INSIGHT: Marginal

JUDGMENT: Fair

ADMISSION LABS/STUDIES:

-COMP-	04/26/23	Range
Na	142	137 - 145
K	3.4L	3.5 - 5.0
CL	110H	98 - 107
CO2	21L	24 - 34
BUN	10	8 - 26
CREAT	1.12	0.57 - 1.25
eGFR	85	
GLUC	94	73 - 115
PROT	8.2	6.0 - 8.3
ALB	4.2	3.5 - 5.0
CA	10.1	8.4 - 10.2
TBIL	0.6	0.1 - 1.1
ALK	104	40 - 150
ALT	23	21 - 72
AST	19	12 - 50
ANION GAP	11.5	8-12

-CBC-	04/26/23	Range
WBC	7.39	4.0 - 11.0
HGB	16.5	12.1 - 17.2
HCT	48.1	38 - 51
PLT	240	130 - 400
MCV	85.0	80 - 100
ANC	3.10	2.0 - 7.0

NEUT% 41.9
LYMPH% 36.4
MONO% 7.6
EOS% 13.3
BASO% 0.5
IGR 0.3 0.0 - 0.5

-ID- 04/26/23 Range
COVID-19 Negative Ref: Negative

-URINE- 04/26/23 Range
PH 6.0 5.0 - 9.0
PROT NEGATIVE Ref: Negative
GLUC NEGATIVE Ref: Negative
KET NEGATIVE Ref: Negative
BILI NEGATIVE Ref: Negative
BLOOD NEGATIVE Ref: Negative
NIT NEGATIVE Ref: Negative
LEUK EST NEGATIVE Ref: Negative

-TOX SCREEN- 04/26/23 Range
AMPHETA Not Detected Ref: Not Detected
BARBIT Not Detected Ref: Not Detected
BENZOD Not Detected Ref: Not Detected
CANNAB DETECTED Ref: Not Detected
COCAINE Not Detected Ref: Not Detected
OPIATES Not Detected Ref: Not Detected

-DRUG LEVEL- 04/26/23 Range
LITHIUM 0.57L 0.60 - 1.20

ADDITIONAL LABS/STUDIES:
A1c 5.7% (4/28)
Triglycerides 136, cholesterol 119, HDL 25, LDL 67 (4/28)

SIGNIFICANT FINDINGS ON DISCHARGE:
MENTAL STATUS EXAM:
Appearance: Adult male in no acute distress, fair grooming, fair attention to hygiene, dressed in hospital scrubs
Behavior: Cooperative, good eye contact eye contact
Speech: Fluent. Non-dysarthric. Occasional stuttering
Mood: "Good"
Affect: Calm, euthymic. Restricted.
Thought processes: Largely linear. Occasionally circumstantial. Concrete.
Thought content: Expresses desire to discharge from hospital to be with his fiancé and receive outpatient care. Denies SI and HI.
Perceptions: Denies all auditory or visual hallucinations.
Insight/judgment: Fair/fair

SUMMARY OF HOSPITAL COURSE:
Patient was admitted voluntarily on 4/26/2023 due to worsening depression with

suicidal ideation, plan, and preparatory behavior, including lying across to rail track in anticipation of an oncoming train. While he was primarily admitted for a depressive episode, suspicion was maintained for a personality contribution. This was reinforced by his rapid improvement without specific intervention. He was maintained on his home lithium, prazosin, and trazodone. Additionally low-dose risperidone was added to target depression, anxiety, and insomnia. Suboxone dosing was not altered during the course of his stay. Initially, the patient appeared labile, tearful, and dysphoric. However, his symptoms quickly improved during hospitalization.

Patient had no acute medical issues over the course of his stay, other than known chronic issues (e.g. chronic lower back pain).

Consults placed: None

Medication changes:

- Risperidone 0.5mg QID PRN for anxiety and sleep

Patient had been taking the Risperidone 0.5mg Q HS in the hospital and did well.

We had discussed continuing the 0.5mg Q HS and to use the 0.5mg Q HS PRN as well.

Family/collateral contact: Fiance (Adrienne Blair) - no safety concerns at time of discharge. He does not have access to any guns or weapons at home. She reports seeing improvement in mood and anxiety during admission

CONDITION ON DISCHARGE:

On the day of discharge, and for at least 3 days prior, the patient denied suicidal ideation or intent. He demonstrated future-oriented thinking, planning to participate in outpatient psychiatric and substance use disorder care. Additionally, he was interested in intensive outpatient treatment, possibly to include recommended dialectical behavioral therapy, as well as couples' counseling with his fiance?. He denied depressed mood. He also denied any hallucinations or delusions.

His risk factors for suicide include mood disorder, multiple suicide attempts, personality traits, concurrent substance use, and psychosocial stressors, including complex grief. However, these risk factors are best addressed by the patient's commitment to participation in outpatient care, ongoing sobriety, and engagement with his support system. It is unlikely that prolonging this hospitalization will do anything more to reduce his suicide risk. His protective factors are noted above, primarily that he is denying depressed mood or suicidal thinking and is demonstrating future-oriented thinking and motivation to seek treatment. He also denies access to guns. At this time, the patient appears to be at low risk for suicide in the imminent future and is appropriate for discharge.

Throughout this hospitalization, the patient denied any homicidal thinking. Homicidal thoughts were not a feature of his original presentation. He does not appear to be at risk for homicide in the imminent future.

Patient completed a Suicide Safety Prevention Plan prior to discharge and was provided with a copy.

Patient was discharged to home on 5/1.

DISCHARGE MEDICATIONS:

Patient: ROCKENHAUS, CONRAD ALAN

A list of reconciled medications was provided to the Veteran/caregiver.

The Veteran/caregiver was counseled on new medications and/or medication changes. Potential risks, benefits, and alternative to medications prescribed were discussed with Veteran/caregiver who was given an opportunity to ask questions, which were answered to the best of my ability and seemingly to their satisfaction. Veteran/caregiver was/were instructed to contact provider (means provided) with any concerns or questions.

INCLUDED IN THIS LIST: Alphabetical list of active outpatient prescriptions dispensed from this VA (local) and dispensed from another VA or DoD facility (remote) as well as inpatient orders (local pending and active), local clinic medications, locally documented non-VA medications, and local prescriptions that have expired or been discontinued in the past 90 days.

NOTE The display of VA prescriptions dispensed from another VA or DoD facility (remote) is limited to active outpatient prescription entries matched to National Drug File at the originating site and may not include some items such as investigational drugs, compounds, etc. NOT INCLUDED IN THIS LIST: Medications self-entered by the patient into personal health records (i.e. My HealtheVet) are not included in this list. Non-VA medications documented outside this VA, remote inpatient orders (regardless of status) and remote clinic medications are NOT included in this list. The patient and provider must always discuss medications the patient is taking, regardless of where the medication was dispensed or obtained. Patient safety alert: Medications and allergies from DOD facilities may be incomplete. Reference JLV for full list.

----- Allergies/ADRs -----

Local Allergies: DARUNAVIR (RASH), SULFA DRUGS (URTICARIA)

Remote Allergies: DARUNAVIR (PRURITUS), NONSTEROIDAL ANTI-INFLAMMATORY (GASTRIC ULCER, ABDOMINAL PAIN), SULFA DRUGS (RASH)

New Medications:

RISPERIDONE 1MG TAB

Take one-half tablet by mouth three times a day as needed for anxiety and difficulty sleeping
Usually Taken for: Mood

VITAMIN B COMPLEX/VITAMIN C CAP/TAB

Take 1 tablet by mouth once daily for vitamin supplement

Usually Taken for: Supplement

The following medications have had a dose change:

BUPRENORPHINE 8MG/NALOXONE 2MG SL TAB

Dissolve 1 tablet under the tongue every morning - this rx for 5-1-23 to 5-16-23

Usually Taken for: Substance Use

Notes: delivered 5/1 at 10:20am

Continue taking these medications at your normal dose:

ACETAMINOPHEN 250/ASA 250/CAFF 65MG TAB

Take 2 tablets by mouth every day as needed for headache

Usually Taken for: Pain

ACETAMINOPHEN 500MG TAB

Take 1 tablet by mouth twice a day as needed for pain

Usually Taken for: Pain

ALBUTEROL 90MCG (CFC-F) 200D ORAL INHL

Inhale 1 puff by mouth every 12 hours as needed for shortness of breath

Usually Taken for: Breathing

ARTIFICIAL SALIVA ORAL SPRAY

Use 2 sprays by mouth four times a day as needed for dry mouth

Usually Taken for: Dry eyes/mouth

ASPIRIN 81MG ENTERIC COATED TAB

Take one tablet by mouth every morning with food to prevent blood clots

Usually Taken for: Blood thinner

ATORVASTATIN CALCIUM 80MG TAB

Take one tablet by mouth at bedtime for cholesterol - avoid grapefruit & grapefruit juice.

Usually Taken for: Cholesterol

CHOLECALCIF 50MCG (D3-2,000UNIT) TAB

Take 1 tablet (50 mcg-2,000 units) by mouth daily for low vitamin d

Usually Taken for: Supplement

DOCUSATE NA 100MG CAP

Take one capsule by mouth twice a day for constipation and stool softener

Usually Taken for: Constipation

FAMOTIDINE 20MG TAB

Take one tablet by mouth twice a day for heartburn

Usually Taken for: Stomach

FLUTICASONE NASAL INH (50MCG, 120 DOSES)

Use 2 sprays in each nostril every morning as needed for nasal allergies

Usually Taken for: Allergies

FOLIC ACID 1MG TAB

Take one tablet by mouth once daily

Usually Taken for: Supplement

GLUCOSE 4GM CHEW TAB

Chew four tablets by mouth as directed as needed for low blood sugar event

Usually Taken for: Diabetes

HYDROXYZINE HCL 25MG TAB

Take one tablet by mouth three times a day as needed for anxiety

Usually Taken for: Sleep/Itching

LEVETIRACETAM 750MG TAB

Take two tablets by mouth twice a day for seizures

Usually Taken for: Neurologic

LITHIUM CARBONATE 300MG CAP

Take four capsules by mouth at bedtime for mental health

Usually Taken for: Mood

Notes: lithium capsules - these replace the lithium tablets from Texas

LORATADINE 10MG TAB

Take one tablet by mouth every morning for allergies

Usually Taken for: Allergies

METFORMIN HCL 500MG TAB

Take one and one-half tablets by mouth twice a day with food for diabetes

Usually Taken for: Diabetes

NALOXONE HCL 4MG/SPRAY SOLN NASAL SPRAY

Use 1 device in one nostril as directed for opioid overdose if opioid overdose suspected. call 911 and follow instructions in kit. may repeat in 2 to 3 minutes if no response.

Usually Taken for: Opioid overdose

NICOTINE 21MG/24HR PATCH

Apply 1 patch externally every morning for smoking cessation. remove at bedtime.

Usually Taken for: Smoking

NICOTINE 4MG GUM

Chew 1 piece (4 mg) by mouth as directed to prevent nicotine cravings every 1-2 hours to prevent nicotine cravings. may use with nicotine patch. do not use gum and lozenge at the same time.

Usually Taken for: Smoking

NICOTINE POLACRILEX 4MG MINI LOZENGE

Dissolve 1 lozenge in mouth as directed to prevent nicotine cravings every 1-2 hours to prevent nicotine cravings. may use with nicotine patch. do not use lozenge and gum at the same time.

Usually Taken for: Smoking

POLYETHYLENE GLYCOL-3350 POWDER

Mix 1 capful in 8 oz water/drink once daily for constipation may repeat dose in the evening if needed.

Usually Taken for: Constipation

PRAZOSIN HCL 1MG CAP

Take three capsules by mouth twice a day for nightmares or sleep disruption

Usually Taken for: Prostate/Nightmares

PREGABALIN 150MG CAP

Take one capsule by mouth three times a day for pain

Usually Taken for: Pain/Neuropathy

TOPIRAMATE 200MG TAB

Take one tablet by mouth twice a day for migraine prevention for migraine prevention

Usually Taken for: Mood/Headaches

TRAZODONE HCL 100MG TAB

Take one tablet by mouth at bedtime for sleep

Usually Taken for: Sleep

Stop taking these medications:

QUETIAPINE FUMARATE 400MG TAB

Take one and one-half tablets by mouth at bedtime for thoughts and mood

Usually Taken for: Mood

----- Scheduled Appointments -----

Visit Date/Time	Visit Information
May 5 2023 9:00AM	ANN SUD-PHD 5
May 12 2023 9:00AM	ANN SUD-PHD 5
May 16 2023 9:30AM	ANN SUD-MD 1
May 16 2023 10:00AM	ANN VVC-VOD SUD-PSY 3
May 18 2023 9:30AM	TEM VVC MED NEURO PHY ESTPT3
May 19 2023 9:00AM	ANN SUD-PHD 5
Jun 13 2023 11:00AM	ANN VVC-VOD NEURO MD-SHATZMAN
Jul 6 2023 9:00AM	CAN WHS ACUPUNCTURE 1
Oct 12 2023 10:00AM	CAN PACT 2 YELLOW-MD 2

Sincerely,
MICHAEL FRANKLIN, MD